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In the Supreme Court of Texas

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals, Fort Worth, Texas
Cause No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court, Tarrant County, Texas

EMERGENCY PETITION FOR WRIT OF PROHIBITION AND EMERGENCY MOTION FOR TEMPORARY RELIEF UNDER TEX. R. APP. P. 52.10

RELATOR REQUESTS ORAL ARGUMENT

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Dated: May 27, 2026

**EMERGENCY MOTION FOR TEMPORARY RELIEF
UNDER TEX. R. APP. P. 52.10**

TO THE HONORABLE SUPREME COURT OF TEXAS:

Relator Kathryn Copeland respectfully moves this Court, under Texas Rule of Appellate Procedure 52.10, for emergency temporary relief prohibiting the Office of the Attorney General from intercepting, collecting, garnishing, seizing, or disbursing Relator's Social Security Disability Insurance (SSDI) backpay to satisfy a child-support arrearage, pending disposition of this original proceeding. Relator shows the following:

I. The OAG has identified this Court as the only court with authority to stop enforcement.

In the Second Court of Appeals, the Office of the Attorney General opposed Relator's emergency motion to stay collection by arguing that Texas Government Code § 22.002(c) deprives intermediate courts of appeals of authority to issue injunctive or stay relief against state agency enforcement actions, and that *only the Texas Supreme Court* may enjoin the OAG under any circumstances. The Second Court of Appeals denied Relator's emergency stay motion by per curiam order on May 13, 2026, without analysis, leaving the OAG's jurisdictional objection unresolved. Relator accepts that argument and presents this petition to the Court the OAG identified as the proper forum.

II. The SSDI backpay is subject to active or imminent interception.

Relator is a licensed attorney appearing pro se who has been adjudicated fully disabled by the Social Security Administration, with an established onset date of June 8, 2023 — a date that predates the May 2024 trial proceedings that produced the support obligation now being enforced. Relator's SSDI benefits represent her primary documented income source presently available. The OAG has asserted in these proceedings its right to collect the child-support arrearage from Relator's SSDI backpay arising from the final order entered on May 31, 2024 in No. 231-686108-20, 231st Judicial District Court, Tarrant County, Texas, the Honorable Judith Wells presiding. Once federal disability backpay is swept and disbursed to a third party, recovery is not available through ordinary appellate relief. The harm is both irreparable and immediate. Under 42 U.S.C. § 407, Congress expressly prohibited SSDI benefits from being subject to 'execution, levy, attachment, garnishment, or other legal process.' The OAG's enforcement action is not merely inequitable — it is prohibited by federal statute.

On May 20, 2026, Relator served on the OAG a written Notice of SSDI Preservation Request and Opportunity to Avoid Emergency SCOTX Filing, requesting within 48 hours either written confirmation that any intercepted SSDI backpay would be preserved and segregated pending court

action, or a 14-day standstill on interception to allow orderly SCOTX presentation. The OAG did not respond by the May 22, 2026 deadline or at all. That notice is attached as Appendix E to this petition.

III. The underlying order is actively contested on appeal.

The child-support arrearage the OAG is enforcing arises from a final SAPCR order entered on May 31, 2024 in the 231st Judicial District Court of Tarrant County, Texas, presently on appeal to the Second Court of Appeals as Cause No. 02-24-00278-CV. See App. G. Relator contends that the trial proceedings from which that order arose were infected by systematic ADA Title II access failures that deprived Relator of meaningful participation in the proceedings that produced the support obligation now being enforced. The State should not permanently sweep a disabled litigant's federal disability backpay while the merits of the underlying order remain under active appellate review.

IV. Emergency relief requested.

Relator respectfully requests that this Court immediately:

- (1) prohibit any interception, garnishment, seizure, offset, disbursement, or collection of Relator's SSDI backpay by the Office of the Attorney General, Child Support Division, or any related enforcement entity;

- (2) order the Office of the Attorney General, Child Support Division, to preserve all intercepted or collected funds in a segregated account pending further order of this Court;
- (3) prohibit disbursement of any intercepted funds to any party while this original proceeding remains pending;
- (4) request an expedited response from Respondents; and
- (5) set this matter for consideration on an emergency basis.

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PETITION FOR WRIT OF PROHIBITION

I. STATEMENT OF THE CASE

This is an original proceeding to prohibit the Office of the Attorney General from enforcing a child-support arrearage against a disabled litigant's federal disability backpay during the pendency of a direct appeal in which the validity of the underlying order is actively contested. The underlying case is a suit affecting the parent-child relationship in the 231st Judicial District Court of Tarrant County, Texas, Cause No. 231-686108-20, the Honorable Judith Wells presiding. The final order was entered on May 31, 2024. Relator's appeal, Cause No. 02-24-00278-CV, is presently pending in the Second Court of Appeals at Fort Worth and raises, among other issues, the validity of the underlying order on ADA Title II and due-process grounds.

During the pendency of that appeal, the OAG has asserted in its filings its right to collect the child-support arrearage from Relator's Social Security Disability Insurance backpay. Relator moved the Second Court of Appeals for a stay of enforcement. The OAG opposed the motion by arguing that Texas Government Code § 22.002(c) deprives intermediate appellate courts of authority to stay enforcement against state agency collection actions, and that *only the Texas Supreme Court* has the power to act. The

OAG's brief states: “[O]nly the Texas Supreme Court may enjoin the Office of the Attorney General under any circumstances.” The Second Court denied the motion by per curiam order on May 13, 2026, without analysis, leaving the OAG's jurisdictional objection unresolved. Relator accepts that argument and presents this petition to the Court the OAG has identified as the proper forum.

II. JURISDICTION

This Court has jurisdiction over original proceedings under Article V, Section 3(a) of the Texas Constitution and Texas Government Code § 22.002. The OAG has specifically invoked § 22.002(c) as the source of this Court's exclusive authority to enjoin the OAG. A writ of prohibition is appropriate because (a) no adequate remedy by appeal exists — the SSDI backpay can be permanently disbursed before the pending appeal is resolved; (b) Relator has no other legal avenue after the intermediate court's per curiam denial; and (c) the OAG is acting in excess of any jurisdiction this Court should recognize it possesses during the pendency of a direct appeal that challenges the validity of the underlying obligation.

III. ISSUES PRESENTED

1. Whether this Court should prohibit the OAG's enforcement of a child-support arrearage against Relator's SSDI backpay while the

underlying order that created the arrearage is on appeal and challenged as the product of ADA access violations that deprived Relator of meaningful trial participation.

2. Whether, when the OAG argues that only this Court has authority to stop its enforcement and the intermediate court declines to act, a writ of prohibition properly issues to preserve the status quo and the res of the appeal pending final resolution.

IV. STATEMENT OF FACTS

A. The trial proceedings and the challenged order.

Relator Kathryn Copeland is a licensed Texas attorney who has been adjudicated fully disabled by the Social Security Administration effective June 8, 2023 — a date that predates the May 2024 trial at which she was required to fully participate. She is the mother of Q.C. and P.C. The SAPCR proceeding was tried in the 231st Judicial District Court of Tarrant County, Cause No. 231-686108-20, the Honorable Judith Wells presiding. Relator contends that throughout the trial proceedings, she was denied meaningful access to the judicial process by ADA Title II violations, including inaccessible facilities, denial of reasonable accommodations, and procedural barriers that prevented her from effectively participating in the proceedings that produced the final order now on appeal.

The final order was entered on May 31, 2024. See App. G. It includes child-support obligations the OAG is presently enforcing. Relator timely filed her notice of appeal and the case is pending in the Second Court of Appeals as Cause No. 02-24-00278-CV.

B. The OAG enforcement action and the Second Court's per curiam denial.

While the appeal has been pending, the OAG has asserted its right to collect the child-support arrearage from Relator's SSDI backpay. Relator moved the Second Court for an emergency stay of that enforcement on the grounds that (a) the underlying order is under active appellate challenge; (b) the support obligation arose from proceedings alleged to involve ADA-access violations; and (c) Relator's SSDI backpay represents her primary documented income source presently available, and disbursement to a third party would constitute irreparable financial harm.

The OAG opposed the stay by invoking Texas Government Code § 22.002(c), arguing that the intermediate court lacked authority to stop agency enforcement and that only the Texas Supreme Court could grant the requested relief. The OAG stated: “[O]nly the Texas Supreme Court may enjoin the Office of the Attorney General under any circumstances.” App. B, at 3. The Second Court denied the stay by per curiam order on May 13, 2026, without analysis, leaving the OAG's jurisdictional objection

unresolved. See App. A. The Second Court also denied Relator's Motion for Temporary Orders and Motion to Seal records by per curiam orders entered the same date. See App. C; App. D. A motion for panel rehearing has been filed in the Second Court preserving Relator's position and requesting that the court either act or expressly document its jurisdictional basis for non-action.

The OAG's enforcement position rests on a child-support order premised on imputed income — income that OAG counsel affirmatively argued at trial was appropriate because Relator was not permanently disabled. OAG trial counsel stated in closing argument: "We heard testimony today that mother is not permanently disabled, even though she argued with me about the definition of disabled. It seems like she's talking disabled when it's convenient for her, or she's not when it's not convenient for her." Stephanie Warnock (OAG), Closing Argument, 20 R.R. 236:3-7 (May 7, 2024). The Social Security Administration subsequently issued a fully favorable decision on April 10, 2026, establishing a disability onset date of June 8, 2023 — nearly one year before that closing argument. See App. F. The enforcement the OAG now pursues is therefore premised on a factual characterization the federal government has formally and retroactively rejected.

C. The broader procedural impasse.

Concurrent with this proceeding, Relator's petition for writ of mandamus in the Fifth Court of Appeals (Cause No. 05-26-00580-CV) was denied on May 11, 2026. That petition challenged the enforcement of a Tarrant County vexatious litigant prefiling order — entered on January 21, 2025, approximately four months after the trial court lost plenary power — as void ab initio under *In re Florance*, 377 S.W.3d 837 (Tex. App.—Dallas 2012). A motion for reconsideration was filed in the Fifth Court on May 27, 2026. The combined effect of these rulings is that Relator is simultaneously blocked from accessing the family court of continuing exclusive jurisdiction to address her children's welfare, and from protecting her disability income from active enforcement of an order whose validity is under appellate review.

D. The children's welfare.

Q.C. (age fourteen, date of birth February 24, 2012) has been formally diagnosed with Avoidant/Restrictive Food Intake Disorder (ARFID) (September 2024) and has presented at the first percentile for height (−2.41 Z-score), meeting WHO stunting criteria, with a resting heart rate of 113 BPM and zero measurable height growth over a critical four-month adolescent development window. P.C. (age ten, turning eleven on June 4,

2026) is the younger child who is underweight at third percentile. A specialized treatment scholarship of approximately \$6,000 was available for Q.C.; the father declined to allow it. The SSDI backpay at issue represents the primary financial resource from which Relator could pursue alternative avenues for her daughters' care. Relator has been judicially found to have net resources effectively zero and to be unable to earn income due to disability — findings accepted by the Second Court. See App. H; App. I. Preservation of that resource pending appellate resolution is independently necessary to protect the welfare of the children whose circumstances the underlying order was entered to serve. On May 4, 2026, Relator's Request for Permission to File an Emergency Motion for Temporary Orders was filed and accepted by the Dallas County 330th District Court — documenting Q.C.'s medical emergency and the exhaustion of available trial-court remedies. See App. K. No ruling has issued.

E. The factual premises of the underlying order have been formally dismantled by subsequent adjudications.

The May 2024 order's child-support provisions rest on a finding of imputed income. At the May 2024 trial, OAG counsel argued in closing that Relator was 'talking disabled when it's convenient' and urged the court to impute income on that basis. The Social Security Administration had already determined, retroactively, that Relator's disability onset was June 8, 2023 —

eight months before that argument was delivered. See App. F. The imputed-income finding was therefore premised on a factual characterization the federal government has since formally established was false at the time it was made.

Three months after the final order, on September 6, 2024, the same trial court found Relator to have net resources effectively zero and to be unable to earn income due to disability. See App. H. The Second Court of Appeals accepted those findings on September 17, 2024. See App. I. A court cannot simultaneously hold that a parent has earning capacity sufficient to establish child-support obligations on imputed income and that the same parent has net resources of zero with disability preventing income. These two findings, by the same court in the same case within months of each other, are irreconcilable. The OAG is enforcing financial obligations premised on the earlier, already-contradicted finding — against the very income source that the disability determination protects.

The same pattern of subsequent vindication extends to Q.C.'s medical circumstances. Throughout the underlying proceedings, Relator raised Q.C.'s declining health and sought treatment. In September 2024 — the same month the indigency finding issued — Q.C.'s own treating pediatrician formally confirmed what Relator had been raising: an ARFID diagnosis,

with objective measurements establishing first-percentile height (−2.41 Z-score in January 2025) meeting WHO stunting criteria, resting heart rate of 113 BPM (April 2026) reflecting malnutrition-associated cardiac stress, and zero measurable height growth over a critical four-month adolescent development window. The clinical facts did not change. They were confirmed. Since the formal diagnosis, the father has continued to refuse to authorize treatment. See App. K (Relator's Dallas 330th emergency filing documenting this ongoing refusal, accepted May 4, 2026; no ruling has issued). The SSDI backpay the OAG seeks to intercept represents the primary resource from which Relator could fund Q.C.'s documented, physician-confirmed medical needs.

The OAG is not enforcing a stable, unchallenged order. It is enforcing, on an emergency basis, financial obligations whose factual premises — the disability characterization, the income imputation, and the children's documented circumstances — have each been formally contradicted by subsequent adjudications: the SSA, the same trial court, and the children's own physician. Texas courts have never held that a state agency may intercept a disabled person's federal disability income, against the express findings of the same court that confirmed that disability, while the merits of the underlying order remain pending before an appellate court.

If this Court declines to act, those funds will be permanently gone before the merits panel has the opportunity to determine whether the order creating the obligation was validly entered in the first place.

V. ARGUMENT

A. Prohibition is the proper remedy because the OAG has no authority to enforce during the pendency of an appeal challenging the validity of the underlying obligation, and no adequate remedy by appeal exists.

A writ of prohibition is appropriate when a party acts in excess of jurisdiction and the relator lacks an adequate remedy by appeal. Here, the “appeal” — the pending Second Court proceeding — is precisely the vehicle the OAG says cannot stop the enforcement. The OAG's own § 22.002(c) position establishes that the normal appellate stay mechanism is unavailable at the intermediate level. A final judgment in Relator's favor on the merits of the appeal will not restore SSDI backpay that has already been intercepted and disbursed to a third party. *In re Prudential Ins. Co.*, 148 S.W.3d 124, 135–36 (Tex. 2004) (mandamus proper when appellate remedy is inadequate because harm cannot be cured by later review). The same principle applies with equal force to a writ of prohibition.

B. The OAG's § 22.002(c) concession directly invokes this Court's jurisdiction and forecloses any argument that Relator has an adequate intermediate remedy.

Texas Government Code § 22.002(c) reserves to this Court certain injunctive authority over state agencies. By invoking § 22.002(c) before the Second Court, the OAG has designated this Court as the proper and exclusive forum for the requested relief. The Second Court's per curiam denial left the OAG's jurisdictional objection unresolved — it neither endorsed nor rejected the OAG's position. The result is that the OAG has told the intermediate court it cannot act and has told Relator to come to this Court. This Court therefore faces a straightforward question: will it exercise the authority the State has already acknowledged this Court possesses? If the answer is no, the consequence is that Texas has no court capable of reviewing OAG enforcement of a disability-based support order while appeal of that order is pending. That is the precedent the OAG's own § 22.002(c) argument creates if this Court declines to act.

C. Enforcement should be prohibited because the arrearage arises from proceedings infected by unresolved ADA-access violations, and the merits of the appeal are not frivolous.

This Court's analysis requires consideration of whether Relator is likely to prevail on the merits of the underlying appeal. *See Tex. R. App. P. 52.10(b)*. Relator's pending Second Court appeal raises ADA Title II access

violations that infected the trial proceedings producing the support obligation now being enforced. The Social Security Administration has adjudicated Relator fully disabled effective June 8, 2023 — eight months before the trial at which she was required to participate without meaningful accommodation. In *Copeland v. Tarrant County*, No. 26-10389 (5th Cir. May 6, 2026), the Honorable Catharina Haynes dissented from denial of emergency relief, independently reviewing the structural ADA non-compliance record and concluding the evidence was sufficient, unrebutted, and supported immediate relief. That independent federal judicial determination establishes that the underlying ADA claims are not frivolous. On May 20, 2026, the Fifth Circuit denied reconsideration; Judge Haynes again noted she would grant relief consistent with her prior footnote. See App. J. The underlying parental-rights and custody determination implicates a constitutionally protected fundamental liberty interest. *Troxel v. Granville*, 530 U.S. 57, 65 (2000) (plurality) (due process ‘protects the fundamental right of parents to make decisions concerning the care, custody, and control of their children’); *Santosky v. Kramer*, 455 U.S. 745, 753 (1982) (parental interest in ‘the companionship, care, custody, and management’ of children is ‘an interest far more precious than any property right’). An order entered in proceedings that denied Relator meaningful access to the courts—and

now enforced against her sole disability income—implicates a confluence of constitutional and statutory violations warranting this Court’s intervention.

The arrearage now being enforced arose from proceedings in which the OAG's own trial counsel argued that Relator's disability was tactical. In closing argument, OAG counsel stated: "We heard testimony today that mother is not permanently disabled, even though she argued with me about the definition of disabled. It seems like she's talking disabled when it's convenient for her, or she's not when it's not convenient for her." Stephanie Warnock (OAG), 20 R.R. 236:3-7 (May 7, 2024). The Social Security Administration subsequently established a disability onset date of June 8, 2023 — eight months before that closing argument was delivered. See App. F. This Court is asked to authorize enforcement of financial obligations premised on a factual characterization the federal government has formally and retroactively rejected. Section 12203(b) prohibits any person from 'coercing, intimidating, threatening, or interfer[ing] with any individual in the exercise or enjoyment of ... any right granted or protected by' the ADA. 42 U.S.C. § 12203(b). The Fifth Circuit has articulated that a § 12203(b) interference claim requires: (1) coercion, intimidation, threats, or interference directed at an individual; (2) on the basis of that individual's exercise or enjoyment of ADA rights; and (3) a causal connection between

the protected right and the defendant's conduct. *Strife v. Aldine Indep. Sch. Dist.*, 138 F.4th 237, 251 (5th Cir. 2025). OAG counsel's closing argument—characterizing Relator as 'talking disabled when it's convenient' eight months before the SSA formally adjudicated her disabled—satisfies each element: the statement targeted her disability status, was directed at her assertion of accommodation rights in the underlying proceedings, and was causally connected to those proceedings. The SSA's subsequent disability adjudication retroactively establishes that the characterization was false when made.

This petition does not ask the Court to decide the merits of the pending SAPCR appeal. It asks only that enforcement of the child-support arrearage against Relator's SSDI backpay be prohibited while the appeal is pending — because the challenged support obligation arose from proceedings in which Relator contends she was denied meaningful access, and because the OAG asserts that only this Court can halt enforcement. The ADA mandates that 'no qualified individual with a disability shall, by reason of such disability, be excluded from participation in or be denied the benefits of the services, programs, or activities of a public entity.' 42 U.S.C. § 12132. This guarantee extends to access to state court proceedings. *Tennessee v. Lane*, 541 U.S. 509, 533–34 (2004). Federal regulations require each public

entity to designate a responsible ADA coordinator and establish a grievance procedure. 28 C.F.R. § 35.107. Tarrant County's own written admissions confirm it maintains no Title II ADA compliance infrastructure for court users and that accommodation decisions are left entirely to individual judges' sole discretion — with no responsive compliance records located. See App. L. The Sixth Circuit has held en banc that the Eleventh Amendment does not bar ADA Title II claims against state domestic relations courts when predicated on a due process theory of exclusion from meaningful participation. *Popovich v. Cuyahoga Cnty. Court*, 276 F.3d 808, 813–16 (6th Cir. 2002) (en banc). Title II's 'participation' requirement directly protects a party's right to a meaningful hearing in child custody proceedings — the precise right at stake here. *Id.* at 816.

D. Irreparable harm to a disabled litigant's primary income source warrants immediate relief.

Federal disability backpay intercepted and disbursed cannot be recovered through later appellate relief. Relator's SSDI benefits are her primary documented income source presently available. Interception of those funds impairs her ability to maintain housing, secure counsel, and address her children's documented medical circumstances. Each day of enforcement without prohibition deepens a harm that no subsequent court order can retroactively remedy. The irreparable nature of the harm is

undisputed — the OAG's own position is that the intermediate court cannot stop it, which is itself an acknowledgment that ordinary appellate processes provide no meaningful protection. The harm has an independent federal statutory dimension. Congress has expressly provided that SSDI benefits ‘shall not be transferable or assignable’ and ‘none of the moneys paid or payable ... shall be subject to execution, levy, attachment, garnishment, or other legal process.’ 42 U.S.C. § 407. The Supreme Court construes § 407 broadly and its exceptions narrowly. *Philpott v. Essex Cnty. Welfare Bd.*, 409 U.S. 413, 415–16 (1973); *Bennett v. Arkansas*, 485 U.S. 395, 396 (1988) (per curiam). Whether the child-support enforcement exception of 42 U.S.C. § 659 extends to a retroactive lump-sum SSDI backpay award—as opposed to ongoing periodic benefit payments—presents a federal statutory preemption question that warrants proper briefing rather than being mooted by premature disbursement of the funds.

The risk calculus is direct. If this Court grants temporary relief and the OAG ultimately prevails on appeal, collection proceeds — the only effect is a brief delay while the appellate process completes. If this Court denies relief and Relator ultimately prevails on appeal, the funds are permanently gone. There is exactly one scenario in which this Court's refusal to act causes irreversible harm: the scenario in which Relator is correct. Temporary

relief pending appellate resolution costs nothing if the OAG is right, and prevents permanent, unrecoverable harm if it is wrong.

PRAYER

For these reasons, Relator Kathryn Copeland respectfully requests that this Court:

- (1) Grant the Emergency Motion for Temporary Relief and immediately prohibit any interception, collection, garnishment, seizure, offset, or disbursement of Relator's SSDI backpay by the Office of the Attorney General, Child Support Division, or any related entity;
- (2) Order preservation of any already-intercepted funds in a segregated account pending further order of this Court;
- (3) Issue the Writ of Prohibition directing that the OAG cease enforcement of the child-support arrearage against Relator's SSDI backpay pending resolution of Cause No. 02-24-00278-CV in the Second Court of Appeals;
- (4) In the alternative, direct the Second Court of Appeals to enter a temporary stay of enforcement pending that court's consideration of Relator's motion for panel rehearing; and
- (5) Grant any additional relief to which Relator may be entitled.

Relator further requests oral argument.

Respectfully Submitted,

Kathryn Copeland

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Dated: May 27, 2026

APPENDIX

Relator designates the following items as the appendix to this petition, as required by Tex. R. App. P. 52.3(k):

- Appendix A:** 2nd Court of Appeals Per Curiam Order (May 13, 2026) — Denial of Emergency Motion to Stay Child Support Enforcement, No. 02-24-00278-CV
- Appendix B:** OAG Response to Appellant's Emergency Motion to Stay Child Support Enforcement, No. 02-24-00278-CV (May 8, 2026) (containing § 22.002(c) argument)
- Appendix C:** 2nd Court of Appeals Per Curiam Order (May 13, 2026) — Denial of Temporary Orders Motion, No. 02-24-00278-CV
- Appendix D:** 2nd Court of Appeals Per Curiam Order (May 13, 2026) — Denial of Motion to Seal, No. 02-24-00278-CV
- Appendix E:** Notice of SSDI Preservation Request and Opportunity to Avoid Emergency SCOTX Filing (May 20, 2026) — 48-hour response deadline; no response received
- Appendix F:** Social Security Administration Fully Favorable Decision (April 10, 2026) establishing onset date June 8, 2023
- Appendix G:** Final Order, No. 231-686108-20, 231st Judicial District Court, Tarrant County, Texas (May 31, 2024) — child-support provisions
- Appendix H:** Order on Supplemental Statement of Inability to Afford Court Costs and Findings of Fact (September 6, 2024), 231st Judicial District Court, Tarrant County, Texas — judicial finding that Relator's net resources are effectively zero and that disability prevents earning income
- Appendix I:** 2nd Court of Appeals Per Curiam Order (September 17, 2024) — accepting trial court indigency findings and ordering Relator may proceed without payment of costs, No. 02-24-00278-CV

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- Appendix J:** 5th Circuit Order Denying Reconsideration of Motion for Injunction Pending Appeal, No. 26-10389, *Copeland v. Tarrant County* (May 20, 2026). Judge Haynes noted she would grant relief consistent with her prior footnote.
- Appendix K:** Petitioner's Request for Permission to File Emergency Motion for Temporary Orders, Dallas County 330th District Court, filed and accepted May 4, 2026. Documents Q.C.'s medical emergency and exhaustion of § 11.102 prefiling restriction remedies.
- Appendix L:** *Copeland v. Tarrant County* — ADA Title II Case Summary and Tarrant County Written Admissions (April–May 2026). County states court-user accommodations are within judges' sole discretion; no Title II compliance records located in response to open-records request.

CERTIFICATION PURSUANT TO TEX. R. APP. P. 52.3(j)

I certify that I have reviewed this petition and concluded that every factual statement in the petition is supported by competent evidence included in the appendix or record.



**KATHRYN COPELAND
RELATOR**

CERTIFICATE OF COMPLIANCE

This petition complies with the word-count limitation of Tex. R. App. P. 9.4(i)(3) because it contains 4,151 words, excluding the parts exempted by Tex. R. App. P. 9.4(i)(1). This document was prepared in Times New Roman 14-point proportionally spaced typeface with double-spaced body text, in compliance with Tex. R. App. P. 9.4(e).



KATHRYN COPELAND

CERTIFICATE OF SERVICE

I certify that on May 27, 2026, a true and correct copy of the foregoing was served on all parties via eFileTexas or verified email, as follows:

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Real Party in Interest — OAG

Office of the Clerk
Second Court of Appeals
Tim Curry Criminal Justice Center
401 W. Belknap, Suite 9000
Fort Worth, Texas 76196


KATHRYN COPELAND

NO. _____

In the Supreme Court of Texas

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals, Fort Worth, Texas
Cause No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court, Tarrant County, Texas

APPENDIX

**TO EMERGENCY PETITION FOR WRIT OF PROHIBITION AND
EMERGENCY MOTION FOR TEMPORARY RELIEF
UNDER TEX. R. APP. P. 52.10**



KATHRYN COPELAND

Texas Bar No. 24086056

Pro Se Relator

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Telephone: (817) 789-8498

Katie@KatieCopeland.com

Dated: May 27, 2026

APPENDIX INDEX

Appendix A

2nd Court of Appeals Per Curiam Order — Denial of Emergency Motion to Stay
Child Support Enforcement (May 13, 2026)
No. 02-24-00278-CV

Appendix B

OAG Response to Appellant's Emergency Motion to Stay Child Support
Enforcement (May 8, 2026)
No. 02-24-00278-CV
[Contains § 22.002(c) argument: "[O]nly the Texas Supreme Court may enjoin
the Office of the Attorney General under any circumstances," at 3]

Appendix C

2nd Court of Appeals Per Curiam Order — Denial of Temporary Orders Motion
(May 13, 2026)
No. 02-24-00278-CV

Appendix D

2nd Court of Appeals Per Curiam Order — Denial of Motion to Seal
(May 13, 2026)
No. 02-24-00278-CV

Appendix E

Notice of SSDI Preservation Request and Opportunity to Avoid
Emergency SCOTX Filing (May 20, 2026)
48-hour response deadline — No response received from OAG

Appendix F

Social Security Administration Fully Favorable Decision (April 10, 2026)
Establishing Disability Onset Date of June 8, 2023

Appendix G

Relevant Child-Support Provisions of Final Order

No. 231-686108-20, 231st Judicial District Court, Tarrant County, Texas

(May 31, 2024)

Appendix H

Order on Supplemental Statement of Inability to Afford Court Costs

and Findings of Fact (September 6, 2024)

No. 231-686108-20, 231st Judicial District Court, Tarrant County, Texas

[Judicial finding: net resources effectively zero; disability prevents earning income]

Appendix I

2nd Court of Appeals Per Curiam Order — Accepting Indigency Findings

and Ordering Appellant May Proceed Without Payment of Costs

(September 17, 2024)

No. 02-24-00278-CV

Appendix J

Fifth Circuit Order Denying Reconsideration (incl. Hon. Catharina Haynes Dissent)

(May 20, 2026)

No. 26-10389

Appendix K

Emergency Motion for Temporary Orders, Dallas County 330th District Court

(May 4, 2026)

No. DF-25-02212

Appendix L

Copeland v. Tarrant County — Fact Sheet with Evidence

(ADA Compliance Failures & Structural Non-Compliance)

UNSWORN DECLARATION PURSUANT TO
TEX. CIV. PRAC. & REM. CODE 132.001

My name is Kathryn Marie Copeland, my date of birth is June 28, 1985 and my current address is 1137 S. Pine Street, Grapevine, Texas 76051 in the United States of America. I declare under penalty of perjury that the foregoing is true and correct and that the Appendixes attached to this filing are true and correct copies of the same. Executed in Tarrant County, State of Texas, on the 27th day of May 2026.



Kathryn Copeland

IN THE SUPREME COURT OF TEXAS

No. _____

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals,
Fort Worth, Texas — No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court,
Tarrant County, Texas

APPENDIX



**Per Curiam Order — Denial of Emergency Motion to Stay
Child Support Enforcement from SSDI Back Pay**

Second Court of Appeals, Fort Worth — No. 02-24-00278-CV

Dated May 13, 2026



**In the
Court of Appeals
Second Appellate District of Texas
at Fort Worth**

No. 02-24-00278-CV

IN THE INTEREST OF Q.C. AND P.C., CHILDREN

On Appeal from the 231st District Court
Tarrant County, Texas
Trial Court No. 231-686108-20

ORDER

We have considered “Appellant’s Emergency Motion to Stay Collection of Child Support Arrearage from SSDI Back Pay Pending Resolution of Appeal.”

The motion is **DENIED**.

We direct the clerk of this court to send a notice of this order to the appellant and the attorneys of record.

Dated May 13, 2026.

Per Curiam

IN THE SUPREME COURT OF TEXAS

No. _____

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals,
Fort Worth, Texas — No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court,
Tarrant County, Texas

APPENDIX

B

**OAG Response to Emergency Motion to Stay
Child Support Enforcement**

Second Court of Appeals, Fort Worth — No. 02-24-00278-CV
(Contains § 22.002(c) argument at page 3)

Dated May 8, 2026

No. 02-24-00278-CV

IN THE
SECOND COURT OF APPEALS
FORT WORTH, TEXAS

IN THE INTEREST OF Q.C. AND P.C., CHILDREN

K.C.
Appellant,

The Office of the Attorney General of Texas
Co-Appellees.

**RESPONSE OF OFFICE OF THE ATTORNEY GENERAL TO
APPELLANT'S EMERGENCY MOTION TO STAY COLLECTION OF
CHILD SUPPORT ARREARAGE FROM SSDI BACK PAY PENDING
RESOLUTION OF APPEAL**

Appellee, the Office of the Attorney General of Texas (OAG), under Tex. R. App. P. 38.6(d) and 10.5(b), files this response to Appellant's Emergency Motion to Stay Collection of Child Support Arrearage from SSDI Back Pay Pending Resolution of Appeal.

STATEMENT OF FACTS

On January 15, 2021, the 231st District Court of Tarrant County, Texas issued an agreed Final Decree of Divorce in which neither parent was ordered to pay child

support. 1 C.R. at 28-63. On April 27, 2021, Appellee C.C. (“C.C.”) filed a Petition to Modify Parent–Child Relationship and alleged a material and substantial change of a child, conservator, or party affected by the agreed divorce decree occurred since the issuance of the agreed divorce decree. 1 C.R. at 66-73; 10 R.R. at 18-20. On September 14, 2022, Appellant filed a First Amended Counter Petition to Modify Parent–Child Relationship and alleged a material and substantial change of a child, conservator, or party affected by the agreed divorce decree occurred since the issuance of the agreed divorce decree. 1 C.R. at 84-97; 10 R.R. at 18-20. On November 13, 2023, the trial court below ordered Appellant to pay \$346.90 in current child support beginning on October 1, 2023. 1 C.R. at 167-201. On February 12, 2024, the trial court below issued an Order to Set Aside Default Judgment and For New Trial in which the trial court vacated the November 13, 2023 modification order and granted a new trial “as to the issues heard on November 13, 2023.” 2 C.R. at 539.

On May 6, 2024, the trial court convened a hearing on the issues relevant to the matter. 19 R.R. at 5. Among the evidence the trial court contemplated when determining Appellant’s child support obligation was Appellant’s testimony. Appellant testified as to the following: She earned \$9750 in fees as an entertainment manager. 13 R.R. at 9. She received \$2500 in grants per semester as a graduate

student. 13 R.R. at 9-10. She received \$20,500 in student loans. 13 R.R. at 12. She worked as a contract paralegal at a wage of \$50 per hour. 13 R.R. at 13-14. She received public benefits on behalf of the children during a period in which she had only two hours of supervised visitation at the courthouse each week. 13 R.R. at 20-21. She explained that she used the public benefits in a manner to receive cash payments. 13 R.R. at 23-24.

Other evidence was also presented to the trial court regarding Appellant's income and net resources. C.C. testified that Appellant previously worked as an attorney and earned \$100,000 per year. 20 R.R. at 96-97. He also testified that Appellant received \$5,300 per month in deposits to her bank account which totaled over \$50,000 in deposits in 2023. 20 R.R. at 96-97.

On May 31, 2024, the trial court below issued an Order in Suit to Modify Parent–Child Relationship in which Mother was ordered to pay \$1,042.60 per month beginning on June 1, 2024. 2 C.R. at 908-936.

ISSUE ONE: This Court lacks subject matter jurisdiction to enjoin the Office of the Attorney General from exercising its administrative authority under federal and state law.

Appellant requests that this Court enjoin the OAG, acting as Title IV-D child support enforcement agency for the State of Texas, from exercising its

administrative authority to garnish or withhold receipts of federal benefits. As explained below, only the Texas Supreme Court may enjoin the OAG under any circumstances.

Subject Matter Jurisdiction to Enjoin Executive Branch Agencies

Texas Government Code section 22.002 states in relevant part:

- (c) Only the supreme court has the authority to issue a writ of mandamus or injunction, or any other mandatory or compulsory writ or process, against any of the officers of the executive departments of the government of this state to order or compel the performance of a judicial, ministerial, or discretionary act or duty that, by state law, the officer or officers are authorized to perform.

TEX. GOV'T. CODE § 22.002 (c); *s Paxton v. Am. Oversight*, 716 S.W.3d 535, 542–43 (Tex. 2025) (citing TEX. CONST. art. IV, § 1 and TEX. GOV'T CODE 22.002(c)) (“In other words, no court, except for the Supreme Court, has that authority. As we have held before, the phrase ‘officers of the executive departments of the government of this state’ includes the officers of the Executive Department enumerated in the Constitution: ‘The Executive Department of the State shall consist of a Governor, who shall be the Chief Executive Officer of the State, a Lieutenant Governor, Secretary of State, Comptroller of Public Accounts, Commissioner of the General Land Office, and Attorney General’”); *see A&T Consultants v. Sharp*, 904 S.W.2d 668, 673–74 (Tex. 1995) (Texas Supreme Court may enjoin all executive officers except governor); *Houston Chronicle Pub. Co. v. Mattox*, 767 S.W.2d 695

(Tex. 1989) (Texas Supreme Court exercises jurisdiction to enjoin both Attorney General and district court over order on open records); *Crowell v. Terrell*, 250 S.W. 252 (Tex. Civ. App.—El Paso 1923, writ ref'd) (El Paso court of appeals held that district court had jurisdiction to enjoin state treasurer because motion of receiver did not seek order compelling state treasurer to perform duty treasurer authorized to perform).

Texas Government Code section 22.002(c) has been invoked in child support cases where the trial court has otherwise violated either Title IV–D of the federal Social Security Act or Title 5 of the Texas Family Code. *See Interest of C.H.*, 13-17-00544-CV, 2019 WL 5251145 (Tex. App.—Corpus Christi Oct. 17, 2019, no pet. h.) (reversing order enjoining OAG from issuing wage withholding; trial court's order also violated chapter 158 of Texas Family Code); *see also Interest of J.G.*, 04-17-00755-CV, 2018 WL 6624509 (Tex. App.—San Antonio Dec. 19, 2018, no pet.) (reversing order enjoining OAG from collecting income tax and directing OAG to deposit funds with court; also a violation of federal law); *see also In re Office of Attorney Gen. of Texas*, 05-18-00086-CV, 2018 WL 1725069 (Tex. App.—Dallas Apr. 10, 2018, no pet.), reh'g denied (May 8, 2018) (denying OAG petition for mandamus in case in which trial court ordered OAG to divulge confidential locate information; held that parent locator service function of Title IV–D agency; granting

OAG petition on issue of order to cease child support distribution on jurisdictional grounds, also trial court violated federal distribution rules); *see also In Interest of H.G.-J.*, 503 S.W.3d 679 (Tex. App.—Houston [14th Dist.] 2016, no pet.) (reversing order instructing OAG to distribute child support payments which would violate federal disbursement rules imposed on OAG as Title IV–D agency by requiring OAG to pay non-obligee third party, here amicus attorney seeking fees); *see also In re C.D.E.*, 533 S.W.3d 367, 370 (Tex. App.—Houston [14th Dist.] 2015, no pet.) (reversing order prohibiting OAG from issuing liens; trial court violated child support lien statute by removing child support lien when obligor had child support arrearages); *see also In re B.N.A.*, 278 S.W.3d 530, 533 (Tex. App.—Dallas 2009, no pet.) (reversing order instructing OAG to distribute child support payments which would violate federal disbursement rules imposed on OAG as Title IV–D agency by requiring OAG to pay non-obligee third party and reversing order enjoining OAG from pursuing additional collection remedies because trial court lacked subject–matter jurisdiction); *see also In re C.J.M.S.*, 269 S.W.3d 206, 208 (Tex. App.—Dallas 2008, pet. denied (reversing order instructing OAG to distribute child support payments which would violate federal disbursement rules imposed on OAG as Title IV–D agency by requiring OAG to pay non-obligee third party); *see also In re A.B., Jr.*, 267 S.W.3d 564 (Tex. App.—Dallas 2008, no pet.) (reversing order instructing

OAG to distribute child support payments which would violate federal disbursement rules imposed on OAG as Title IV–D agency by requiring OAG to pay non-obligee third party).

This Court lacks subject matter jurisdiction to enjoin the OAG under these facts. As explained below, any possible garnishment or withholding of receipt of federal benefits is within the sole administrative authority of the OAG.

Administrative Authority Under Federal and State Law

The OAG is the designated Title IV–D child support enforcement agency for the State of Texas. TEX. FAM. CODE §§ 231.001, 231.101; 42 U.S.C. § 654 (authorization for States to create, designate state child support enforcement agency through “State Plan”). As the Title IV–D agency, the OAG may exercise administrative authority under federal law to garnish moneys paid by the United States. TEX. FAM. CODE § 231.101; 42 U.S.C. § 659(a), (b).

The OAG, as Title IV–D agency, may issue an administrative writ of withholding “at any time until all current support, including medical support and dental support, child support arrearages, and Title IV–D service fees authorized under [Texas Family Code] Section 231.103 for which the obligor is responsible have been paid.” TEX. FAM. CODE § 158.502(a); *see also Atty. Gen. v. Redding*, 60 S.W.3d 891, 894 (Tex. App.—Dallas 2001, no pet.) (“At the same time, the

legislature created a new remedy for the collection of child support and arrearages—the administrative writ of withholding. Like the judicial writ, the administrative writ could be brought at any time until all arrearages have been paid.”); *see also In re A.D.*, 73 S.W.3d 244, 246 (Tex. 2002) (“Like an order holding the obligor in contempt, a wage-withholding order is available to remedy past violations of a support order whether or not the court has reduced the delinquent amount to a single, cumulative judgment.”).

The Code of Federal Regulations expressly permits state child support enforcement agencies to collect interest on child support arrears via withholding Social Security under certain conditions. First, Part 581 of Title 5 of the Code of Federal Regulations states that each “governmental entity” under the authority of the federal government which disburses money to “any individual” shall be subject to all requirements of withholding for child support enforcement as if the governmental entity “were a private person”. 5 C.F.R. §§ 581.101 (describing purpose of Part 581 of the Code of Federal Regulations and cited 42 U.S.C. § 659 (federal Social Security Act)), 581.102(b) (definition of “governmental entity”).

Second, Part 581 of Title 5 of the Code of Federal Regulations specifically cites Social Security benefits as subject for garnishment or withholding, as stated in relevant part here:

(c) For obligors generally:

(1) Periodic benefits, including a periodic benefit as defined in section 428(h)(3) of title 42 of the United States Code, title II of the Social Security Act, to include a benefit payable in a lump sum if it is commutation of, or a substitute for, periodic payments; *or other payments to these individuals under the programs established by subchapter II of chapter 7 of title 42 of the United States Code (Social Security Act)*; and payments under chapter 9 of title 45 of the United States Code (Railroad Retirement Act) or any other system, plan, or fund established by the United States (as defined in section 662(a) of title 42 of the United States Code) which provides for the payment of:

(i) Pensions;

(ii) Retirement benefits;

(iii) Retired/retainer pay;

(iv) Annuities; and

(v) Dependents' or survivors' benefits when payable to the obligor;

5 C.F.R. § 581.103(c) (emphasis added).

Finally, Part 581 of Title 5 of the Code of Federal Regulations permits child support enforcement agencies to request withholding of Social Security benefits for accrued interest on child support arrears if the interest is deemed “as child support”, as stated here:

Before complying with legal process that requires withholding for the payment of attorney fees, interest, and/or court costs, the governmental entity must determine that the legal process meets both of the following requirements:

- (a) The legal process must expressly provide for inclusion of attorney fees, interest, and/or court costs as (rather than in addition to) child support and/or alimony payments;
- (b) The awarding of attorney fees, interest, and/or court costs as child support and/or alimony must be within the authority of the court, authorized official, or authorized State agency that issued the legal process. It will be deemed to be within the authority of the court, authorized official, or authorized State agency to award attorney fees as child support and/or alimony if such order is not in violation of or inconsistent with State or local law, even if State or local law does not expressly provide for such an award.

5 C.F.R. § 581.307.

In sum, the OAG has sole administrative authority to garnish or withhold any receipt of federal benefits under the facts here. Coupled with this Court's lack of subject matter jurisdiction to enjoin the OAG, this Court should deny Appellant's motion.

ISSUE TWO: Appellant has not met her burden to show that enforcement of the modification order should be stayed.

Texas Family Code section 109.002 states in relevant part:

- (c) An appeal from a final order, with or without a supersedeas bond, does not suspend the order unless suspension is ordered by the court rendering the order. The appellate court, on a proper showing, may permit the order to be suspended, unless the order provides for the termination of the parent-child relationship in a suit brought by the state or a political subdivision of the state permitted by law to bring the suit.

TEX. FAM. CODE § 109.002(c); *McGee v. Ponthieu*, 634 S.W.2d 780, 782 (Tex. App.—Amarillo 1982, no writ) (appeals court examining language of predecessor statute to Texas Family Code section 109.002 using same phrase “the appellate court, on a proper showing, may permit the judgment to be suspended” in its pre-1995 recodification form).

In *McGee*, grandparents filed a petition for writ of mandamus with the appellate court seeking to stay the execution of a judgment on conservatorship issues after the trial court refused to suspend the judgment. 634 S.W.2d at 781–82. The court of appeals held that:

The authorization for suspension of the judgment by the rendering court, it has been held, vests in that court the exercise of discretion in determining whether the judgment should be suspended. By the authorization language, the same discretion may be exercised by the appellate court upon a proper showing. Necessarily, then, the requirement of a *proper showing obligates a relatorap to demonstrate that the trial court clearly abused its discretion in refusing to suspend its judgment on presentation of essentially the same matters offered to the appellate court.*

Id. (emphasis added); *see also Ross v. Brown*, 491 S.W.2d 690, (Tex. Civ. App.—Tyler 1973, writ ref’d n.r.e.) (“The rule seems to be one of long standing in Texas that an action in equity to set aside an execution sale must be brought in the court out of which the writ was issued. This is the rule even though the validity of the

judgment is not questioned. A suit for an injunction to stay execution of a judgment must be tried in the court where the judgment was rendered.”) (internal citations omitted).

Simply put, the trial court must first determine whether the judgment must be suspended. If the trial court refuses to suspend the judgment, the party may then seek appellate relief. Here, there is a limited record on whether Appellant requested that the trial court stay its enforcement of its own order. *See* 2 C.R. at 836-844 (Appellant’s Objection to Petitioner’s Motion to Enter Final Order in Suit to Modify Parent–Child Relationship and Related Motions and Notice of Non-Suit Without Prejudice in which Appellant requested stay of trial court proceedings due to lack of disability accommodations). Based on Appellant’s motion before this Court, there is no citation to the record indicating that Appellant specifically requested that the trial court stay enforcement of the child support provisions of its order. Nonetheless, as stated in Issue One, the OAG has independent administrative authority under federal and state law to enforce the child support obligation administratively. This Court should deny Appellant’s motion.

CONCLUSION AND PRAYER

The OAG respectfully requests that this Court deny Appellant’s Emergency Motion To Stay Collection Of Child Support Arrearage From SSDI Back Pay

Pending Resolution Of Appeal.

Respectfully submitted,

Ken Paxton,
Attorney General of Texas

Brent Webster
First Assistant Attorney General

/s/ Deterrean Gamble
Deterrean Gamble
Assistant Attorney General
State Bar No. 24062194
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Attorneys for Appellee
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Certificate of Compliance

I certify that this document complies with the typeface requirements of Tex. R. App. P. 9.4(e) because it has been prepared in a conventional typeface no smaller than 14-point for text and 12-point for footnotes. This document also complies with the word-count limitations of Tex. R. App. P. 9.4(i), if applicable, because it contains 2656 words, excluding any parts exempted by Tex. R. App. P. 9.4(i)(1).

/s/ Deterrean Gamble
Deterrean Gamble
Assistant Attorney General

Certificate of Service

I certify that a copy of this **RESPONSE OF OFFICE OF THE ATTORNEY GENERAL TO APPELLANT'S EMERGENCY MOTION TO STAY COLLECTION OF CHILD SUPPORT ARREARAGE FROM SSDI BACK PAY PENDING RESOLUTION OF APPEAL** was served on the parties or their counsel via e-filing pursuant to the Texas Rules of Civil Procedure on Friday, May 8, 2026.

/s/ Deterrean Gamble

Deterrean Gamble
Assistant Attorney General

K.C.
Appellant

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Amicus Attorney

Automated Certificate of eService

This automated certificate of service was created by the eFiling system. The filer served this document via email generated by the eFiling system on the date and to the persons listed below. The rules governing certificates of service have not changed. Filers must still provide a certificate of service that complies with all applicable rules.

Office Filer Appellate on behalf of Deterrean Gamble

Bar No. 24062194

CSD-Appeals@texasattorneygeneral.gov

Envelope ID: 114682202

Filing Code Description: Original Proceeding Response

Filing Description: Response to Appellant's Emergency Motion to Stay

Child Support Collection

Status as of 5/11/2026 7:48 AM CST

Case Contacts

Name	BarNumber	Email	TimestampSubmitted	Status
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Katherine Allen	24040413	kallen@allenweaver.com	5/8/2026 5:12:27 PM	SENT
André L. Smith		paralegal@susansmithfamilylaw.com	5/8/2026 5:12:27 PM	SENT
K. C.		kcopelandlaw@gmail.com	5/8/2026 5:12:27 PM	SENT
Deterrean Gamble		deterrean.gamble@oag.texas.gov	5/8/2026 5:12:27 PM	SENT
Nicole Loya		nicole.loya@oag.texas.gov	5/8/2026 5:12:27 PM	SENT
Matthew Deal		matthew.deal@texasattorneygeneral.gov	5/8/2026 5:12:27 PM	SENT
Christopher Copeland		cjcopeland@gmail.com	5/8/2026 5:12:27 PM	SENT
John Allen Douglas		John@JADouglasLaw.com	5/8/2026 5:12:27 PM	SENT

IN THE SUPREME COURT OF TEXAS

No. _____

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals,
Fort Worth, Texas — No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court,
Tarrant County, Texas

APPENDIX

C

**Per Curiam Order — Denial of
Motion for Temporary Orders**

Second Court of Appeals, Fort Worth — No. 02-24-00278-CV

Dated May 13, 2026



**In the
Court of Appeals
Second Appellate District of Texas
at Fort Worth**

No. 02-24-00278-CV

IN THE INTEREST OF Q.C. AND P.C., CHILDREN

On Appeal from the 231st District Court
Tarrant County, Texas
Trial Court No. 231-686108-20

ORDER

We have considered “Appellant’s Motion for Temporary Orders Pending Appeal” and “Appellant’s Notice of Supplemental Evidence in Support of Motion for Temporary Orders Pending Appeal.”

The motion is **DENIED**.

We direct the clerk of this court to send a notice of this order to the appellant and the attorneys of record.

Dated May 13, 2026.

Per Curiam

IN THE SUPREME COURT OF TEXAS

No. _____

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals,
Fort Worth, Texas — No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court,
Tarrant County, Texas

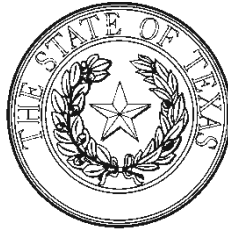
APPENDIX

D

**Per Curiam Order — Denial of
Motion to Seal**

Second Court of Appeals, Fort Worth — No. 02-24-00278-CV

Dated May 13, 2026



**In the
Court of Appeals
Second Appellate District of Texas
at Fort Worth**

No. 02-24-00278-CV

IN THE INTEREST OF Q.C. AND P.C., CHILDREN

On Appeal from the 231st District Court
Tarrant County, Texas
Trial Court No. 231-686108-20

ORDER

We have considered “Appellant’s Motion to File Medical Records Under Seal.”

The motion is **DENIED**.

We direct the clerk of this court to send a notice of this order to the appellant and the attorneys of record.

Dated May 13, 2026.

Per Curiam

IN THE SUPREME COURT OF TEXAS

No. _____

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals,
Fort Worth, Texas — No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court,
Tarrant County, Texas

APPENDIX

E

**Notice of SSDI Preservation Request and
Opportunity to Avoid Emergency SCOTX Filing**

OAG Child Support Division — 48-hour response deadline; no response received

Dated May 20, 2026



KATHRYN COPELAND
(817) 789-8498
KATIE@KATIECOPELAND.COM

May 20, 2026

Via Email — deterrean.gamble@oag.texas.gov
Via Email — nicole.loya@oag.texas.gov
Via Email — matthew.deal@texasattorneygeneral.gov

Deterrean Gamble, Assistant Attorney General
Nicole Loya, Assistant Attorney General
Office of the Attorney General of Texas — Child Support Division
P.O. Box 12017, Austin, Texas 78711-2017

Re: In the Interest of Q.C. and P.C., Children
Second Court of Appeals No. 02-24-00278-CV
Trial Court Cause No. 231-686108-20, 231st JDC, Tarrant County
**Notice of SSDI Preservation Request and Opportunity to Avoid
Emergency SCOTX Filing**

Counsel:

This letter is not a meet-and-confer. It is notice and a documented opportunity to avoid an emergency filing in the Texas Supreme Court, including an application for temporary relief under Texas Rule of Appellate Procedure 52.10.

I write regarding OAG's May 8, 2026 Response filed in No. 02-24-00278-CV, in which the OAG argued that the Second Court of Appeals lacks subject matter jurisdiction to enjoin the OAG under Texas Government Code § 22.002(c) and that "only the Texas Supreme Court"

has authority to provide SSDI intercept relief. The Second Court denied my Emergency Motion to Stay Collection of Child Support Arrearage from SSDI Backpay without analysis on May 13, 2026. The OAG's position and the Second Court's refusal together confirm that the Texas Supreme Court is the only available forum — which is precisely where I intend to file.

1. The Narrow Preservation Request

Before filing at SCOTX, I am asking one thing: will the OAG agree to a narrow, time-limited preservation and standstill, so that irreversible harm does not occur before the only court with jurisdiction can act?

Specifically, I request that the OAG do one of the following within 48 hours:

- A. Confirm in writing that any SSDI backpay received or intercepted in this matter will be preserved and segregated — not disbursed — pending further order of the Texas Supreme Court or the Second Court of Appeals; or
- B. Confirm that the OAG will refrain from initiating or processing any interception of SSDI backpay for a period of 14 days from the date of this letter, to allow orderly presentation of the jurisdictional issue to the Texas Supreme Court.

I am not asking the OAG to waive any enforcement authority, concede any legal argument, or forgo collection permanently. I am asking for a narrow, documented pause to allow the court the OAG itself identified as the proper forum to act before the harm is irreversible.

2. Why Refusal Creates Unnecessary Urgency

SSDI backpay, once intercepted and applied to an arrearage, cannot be recovered through ordinary appellate process. If the OAG declines this standstill request and the funds are intercepted before SCOTX can act, the emergency motion will necessarily document that the OAG — after itself arguing that SCOTX is the only proper forum — declined a narrow, 14-day pause to allow that forum to act. That is not a record position the OAG should want to be in.

A 14-day pause costs the OAG nothing, because if SCOTX denies relief, collection proceeds. If SCOTX grants relief, the funds are preserved. There is no equitable argument for refusing a standstill under these circumstances.

3. Record Integrity Context (Notice Only — No Response Required Now)

The OAG's May 8, 2026 response relies on trial record characterizations of my financial situation and ability to pay. That record was formed at proceedings in which OAG counsel appeared and cross-examined me directly on disability status and accommodation requests. The record includes the following statement by OAG counsel in closing argument:

"We heard testimony today that mother is not permanently disabled, even though she argued with me about the definition of disabled. It seems like she's talking disabled when it's convenient for her, or she's not when it's not convenient for her."

— Stephanie Warnock (OAG), Closing Argument, 20 R.R. 236:3–7
(May 7, 2024)

This characterization was placed in the record after OAG counsel repeatedly interrupted my attempts to explain the medical context during cross-examination (20 R.R. 210–211). The Social Security Administration's fully favorable determination (April 10, 2026) established disability onset of June 8, 2023 — a date predating the May 2024 trial — which is directly inconsistent with the trial record characterization on which OAG now relies.

I am not asking the OAG to respond to this in this letter. A separate demand to cure will follow, specifically addressing disability-related record misstatements and ADA interference under 42 U.S.C. § 12203(b). This notice is included here so the OAG understands the full scope of the record integrity issues at stake before deciding whether to agree to a 14-day standstill.

4. Preservation Notice

Please immediately preserve all records relating to:

- A. Decisions, policies, and communications regarding interception or collection of SSDI backpay in Cause No. 231-686108-20 / No. 02-24-00278-CV;
- B. Communications with C.C. or his counsel regarding enforcement timing, strategy, or collection priorities in this case;

C. Internal drafts and communications related to the May 8, 2026 response, including any analysis of the § 22.002(c) jurisdictional argument;

D. All communications related to ADA accommodation requests or disability-related assertions in this matter, including hearing preparation materials and internal correspondence related to the May 2024 trial proceedings.

5. No Waiver

Nothing in this letter waives any claims, arguments, defenses, objections, appellate issues, bill-of-review grounds, or federal remedies. This communication is made for settlement and mitigation purposes only, and is without prejudice to any right or claim.

Please respond by Friday, May 22, 2026 at 10:00 a.m. Central Time. I will file at SCOTX regardless of your response, but your response — or non-response — will be part of the record presented to the Court regarding whether emergency temporary relief is necessary to prevent irreversible harm pending SCOTX's consideration.

Respectfully,



KATHRYN M. COPELAND

Pro Se Appellant

Texas Bar No. 24086056

1137 S. Pine Street, Grapevine, Texas 76051

(817) 789-8498 | Katie@KatieCopeland.com

IN THE SUPREME COURT OF TEXAS

No. _____

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals,
Fort Worth, Texas — No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court,
Tarrant County, Texas

APPENDIX

F

Social Security Administration

Fully Favorable Decision

Disability Onset Date Established: June 8, 2023

Dated April 10, 2026



Office of Hearings Operations
ROOM 9A27
819 TAYLOR STREET
FORT WORTH, TX 76102-6124

Date: April 10, 2026

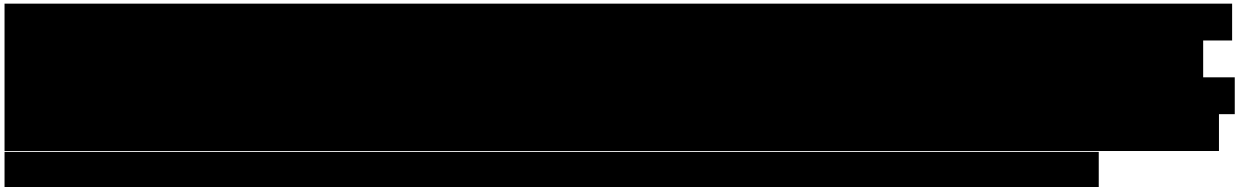
Kathryn Marie Copeland



Notice of Decision – Fully Favorable

I carefully reviewed the facts of your case and made the enclosed fully favorable decision. Please read this notice and my decision.

Although my decision is fully favorable, you have the right to an oral hearing and to examine the evidence on which I based my decision. Please contact the office listed above if you want to have an oral hearing or examine the evidence in your case record.



See Next Page

[REDACTED]

[REDACTED]
[REDACTED]
[REDACTED]
[REDACTED]

[REDACTED]

[REDACTED]
[REDACTED]
[REDACTED]

[REDACTED]
[REDACTED]

[REDACTED]

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[REDACTED]

[REDACTED]

[REDACTED]
[REDACTED]
[REDACTED]

If You Have Any Questions

1. Visit www.ssa.gov for fast, simple, and secure online service.
2. Call us at **1-800-772-1213**, weekdays from 8:00 am to 7:00 pm. If you are deaf or hard of hearing, call TTY **1-800-325-0778**. Please mention this notice and decision when you call.
3. You may also call your local office at (866) 704-4858.

SOCIAL SECURITY
RM 1-A-07 FEDERAL BLDG
819 TAYLOR ST
FORT WORTH, TX 76102-9844

Laura Roberts
Administrative Law Judge

Enclosures:
Decision Rationale

[REDACTED]

cc: SUSAN HARRIET BAXTER
600 EAST WEATHERFORD
FORT WORTH, TX 76102-3264

**SOCIAL SECURITY ADMINISTRATION
Office of Hearings Operations**

DECISION

IN THE CASE OF

Kathryn Marie Copeland
(Claimant)

(Wage Earner)

CLAIM FOR

Period of Disability, Disability Insurance
Benefits, and Supplemental Security Income

[REDACTED]
(Beneficiary Notice Control Number)

Social Security Number removed for your protection

JURISDICTION AND PROCEDURAL HISTORY

This case is before the undersigned on a request for hearing dated September 15, 2025 (20 CFR 404.929 *et seq.* and 416.1429 *et seq.*). The evidence of record supports a fully favorable decision; therefore no hearing has been held (20 CFR 404.948(a) and 416.1448(a)). The claimant is represented by Susan Baxter, a non-attorney representative (Exhibit 20B).

The claimant has amended the alleged onset date of disability to June 8, 2023 (Exhibit 13D).

FINDINGS OF FACT AND CONCLUSIONS OF LAW

After careful consideration of the entire record, the undersigned makes the following findings:

■ [REDACTED]

■ [REDACTED]
[REDACTED]
[REDACTED].

3. The claimant has the following severe impairments: neoplasm of the brain, long haul Covid, hiatal hernia, postural orthostatic tachycardia syndrome (POTS), Ehlers-Danlos Syndrome (EDS), polyarthralgia, attention deficit hyperactivity disorder (ADHD), [REDACTED]

[REDACTED], intracranial hypotension, [REDACTED]
[REDACTED]

(20 CFR 404.1520(c) and 416.920(c)).

■ [REDACTED]
[REDACTED]
[REDACTED]
[REDACTED]

See Next Page

5. [REDACTED]

She can tolerate no concentrated exposure to dust, fumes, odors, gases or other pulmonary irritants. The claimant can understand, remember and carry out detailed, but not complex, instructions. She will be off task 20% of the time, she will need one hour of additional breaks, and she will miss two days of work per month.

In making this finding, the undersigned has considered all symptoms and the extent to which these symptoms can reasonably be accepted as consistent with the objective medical evidence and other evidence, based on the requirements of 20 CFR 404.1529 and 416.929 and SSR 16-3p. The undersigned also considered the medical opinion(s) and prior administrative medical finding(s) in accordance with the requirements of 20 CFR 404.1520c and 416.920c.

The claimant has the following degree of limitation in the four broad areas of mental functioning set out in the disability regulations for evaluating mental disorders and in the mental disorders listings in 20 CFR, Part 404, Subpart P, Appendix 1: a moderate limitation in understanding, remembering, or applying information; a mild limitation in interacting with others; a moderate limitation in concentrating, persisting, or maintaining pace; and a moderate limitation in adapting or managing oneself.

[REDACTED]

After careful consideration of the evidence, the undersigned finds that the claimant's medically determinable impairments could reasonably be expected to cause the alleged symptoms. The claimant's statements concerning the intensity, persistence and limiting effects of these symptoms are reasonably consistent with the medical evidence and other evidence in the record for the reasons explained in this decision.

The intensity of the claimant's physical and cognitive symptoms, and the extent of the alleged functional limitations, are supported by objective findings and the record of medical treatment.

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

The description of the symptoms and limitations which the claimant has provided throughout the record has generally been consistent and persuasive. Neurology reports from 2025 and 2026 document that the claimant experienced worsening neurological symptoms, severe tremors, postural headache, dizziness and brain fog (Exhibit 68F). She was diagnosed with intracranial hypotension in the setting of EDS and prior trauma, for which additional testing was recommended to assess a possible cerebrospinal fluid leak (Exhibit 68F/10). [REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

The claimant's allegations are supported by multiple doctor statements. Primary care doctor, John Bui M.D. submitted a statement in September 2023 noting diagnoses of ADHD, Covid Long Hauler, POTS disease, and Vestibular schwannoma (Exhibit 25F/2). He noted symptoms of fatigue, poor focus, confusion, dizziness and memory issues, and opined that the claimant required learning accommodations such as a flexible schedule, and more time to complete assignments. In March 2025, Dr. Bui submitted a statement noting diagnoses of vestibular schwannoma, EDS, post covid, and MALS. He opined that the claimant was unable to stand or walk for extended periods due to orthostatic intolerance and EDS-related joint instability; she could not maintain seated positions for extended periods without symptom exacerbation; and she required frequent position changes and rest periods throughout the day. He noted that the claimant experienced unpredictable symptom flares requiring complete rest; she could not reliably sustain concentration for work-related tasks; and she experienced significant post-exertional malaise following minimal physical or cognitive exertion (Exhibit 44F/68). The undersigned finds these opinions persuasive, as they are supported by the objective evidence of joint instability and cognitive impairment.

[REDACTED]

[REDACTED]

[REDACTED]

Dr. Kayster submitted a statement in February 2026 and notes diagnoses of intracranial hypotension and cognitive impairment, with dizziness, imbalance, and positional neurologic symptoms that worsened with upright posture (sitting/standing) and improved with recumbency (Exhibit 69F). He noted objective findings of slowed responses and delayed recall; deficits in sustained attention, selective attention, processing speed, and speeded word retrieval; and abnormal brain MRI findings and neuro-ophthalmology testing. He opined that the claimant required frequent recumbency throughout the day and she could not sustain predictable function for prolonged periods. He found the claimant could sit, stand or walk for less than 2 hours in an 8-hour workday, and she demonstrated marked limitations in areas of memory, concentration and adaptation. The undersigned finds this opinion persuasive as it is supported by objective findings and the history of treatment.

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

In summary, the undersigned finds the objective findings are consistent with, and support a limitation to, sedentary exertional activities. The claimant's chronic fatigue, widespread pain, weakness and instability, limit her to lifting up to 10 pounds, limit her to standing/walking for 2 hours in an 8-hour workday and prevent her from crouching, crawling or climbing ladders, ropes or scaffolds. She can occasionally climb ramps/stairs, balance, stoop, and kneel, to avoid exacerbations of pain or fatigue. Due to severe orthostatic intolerance, she cannot perform work involving any exposure to extreme hot temperatures (defined as over 90 degrees), extreme cold temperatures (defined as under 30 degrees), excessive humidity (defined as over 80%), dangerous moving machinery, unprotected heights or vibrations as defined by the SCO. She can tolerate no concentrated exposure to dust, fumes, odors, gases or other pulmonary irritants

because of residual Covid symptoms. The claimant can understand, remember and carry out detailed, but not complex, instructions, to account for moderate limitations in her ability to understand, remember and apply information, concentrate, persist or maintain pace, and adapt and manage herself. She will be off task 20% of the time, she will need one hour of additional breaks, and she will miss two days of work per month, because of intracranial hypotension and cognitive impairment.

[REDACTED]

[REDACTED]

[REDACTED]

[REDACTED]

10. Considering the claimant's age, education, work experience, and residual functional capacity, there are no jobs that exist in significant numbers in the national economy that the claimant can perform (20 CFR 404.1560(c), 404.1566, 416.960(c), and 416.966).

[REDACTED]

11. The claimant has been under a disability as defined in the Social Security Act since June 8, 2023, the amended alleged onset date of disability (20 CFR 404.1520(g) and 416.920(g)).

DECISION

Based on the application for a period of disability and disability insurance benefits filed on May 28, 2023, the claimant has been disabled under sections 216(i) and 223(d) of the Social Security Act since June 8, 2023.

[REDACTED]

[REDACTED]
[REDACTED]
[REDACTED]
[REDACTED]

/s/ *Laura Roberts*

Laura Roberts
Administrative Law Judge

April 10, 2026

Date

IN THE SUPREME COURT OF TEXAS

No. _____

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals,
Fort Worth, Texas — No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court,
Tarrant County, Texas

APPENDIX

G

**Relevant Child-Support Provisions
of Final Order**

No. 231-686108-20, 231st Judicial District Court,
Tarrant County, Texas
Dated May 31, 2024

13. CHILD SUPPORT

13.1 Monthly Payments

IT IS ORDERED that KATHRYN COPELAND is obligated to pay and shall pay to CHRISTOPHER J. COPELAND child support of **Three Hundred Forty-Six and 90/100ths Dollars (\$346.90)** per month, with the first payment due and payable on October 1, 2023, and an identical payment of **Three Hundred Forty-Six and 90/100ths Dollars (\$346.90)** due and payable on the first day of each month through May 1, 2024.

Thereafter, IT IS ORDERED that KATHRYN COPELAND is obligated to pay and shall pay to CHRISTOPHER J. COPELAND child support of **One Thousand Forty-Two and 60/100ths Dollars (\$1,042.60)** per month, with the first payment due and payable on June 1, 2024, and an identical payment of **One Thousand Forty-Two and 60/100ths Dollars (\$1,042.60)** due and payable on the first day of each month thereafter until the first month following the date of the earliest occurrence of one of the events specified below:

1. any child reaches the age of eighteen years or graduates from high school, whichever occurs later, subject to the provisions for support beyond the age of eighteen years set out below;
2. any child marries;
3. any child dies;
4. any child enlists in the armed forces of the United States and begins active service as defined by section 101 of title 10 of the United States Code; or
5. any child's disabilities are otherwise removed for general purposes.

Thereafter, when [REDACTED] COPELAND has legally emancipated, KATHRYN COPELAND is ORDERED to pay to CHRISTOPHER J. COPELAND child support of **Eight Hundred Thirty-Four and 08/100ths Dollars (\$834.08)** per month, due and payable on the first day of the first month immediately following the date of the earliest occurrence of one of the events specified above for that child and an identical sum of **Eight Hundred Thirty-Four and 08/100ths Dollars (\$834.08)** due and payable on the first day of each month thereafter until the next occurrence of one of the events specified above for [REDACTED] COPELAND for whom KATHRYN COPELAND remained obligated to pay support under this order.

If a child is eighteen years of age and has not graduated from high school and KATHRYN COPELAND's obligation to support the child has not already terminated, IT IS ORDERED that KATHRYN COPELAND's obligation to pay child support to CHRISTOPHER J. COPELAND shall not terminate but shall continue for as long as the child is enrolled—



1. under chapter 25 of the Texas Education Code in an accredited secondary school in a program leading toward a high school diploma or under section 130.008 of the Education Code in courses for joint high school and junior college credit and is complying with the minimum attendance requirements of subchapter C of chapter 25 of the Education Code or
2. on a full-time basis in a private secondary school in a program leading toward a high school diploma and complying with the minimum attendance requirements imposed by that school.

13.2 *Statement on Guidelines*

In accordance with Texas Family Code section 154.130, the Court makes the following findings and conclusions regarding this child support order:

1. The net resources of KATHRYN COPELAND per month are \$4,170.40.
2. The net resources of CHRISTOPHER J. COPELAND per month are \$5,107.62.
3. The number of children before the Court is two (2).
4. The names and dates of all children not before the court who resides in the same household with CHRISTOPHER J. COPELAND and/or KATHRYN COPELAND for which either parent has a legal duty to pay is: None
5. The amount of child support payments per month that is computed if Section 154.125, Texas Family Code, is applied to the first \$9,000 of KATHRYN COPELAND's net resources for child support is \$1,042.60.
6. The percentage applied to the first \$9,000.00 of KATHRYN COPELAND's net resources for child support by the actual order rendered by the Court is 25.
7. The specific reasons that the amount of support per month ordered by the Court varies from the amount computed by applying the percentage guidelines of section 154.125 of the Texas Family Code are: None. The percentage is within guidelines.

13.3 *Withholding from Earnings*

IT IS ORDERED that any employer of KATHRYN COPELAND shall be ordered to withhold the child support payments ordered in this order from the disposable earnings of KATHRYN COPELAND for the support of [REDACTED] N [REDACTED] E COPELAND and [REDACTED] COPELAND .

13.4 *Withholding as Credit against Support Obligation*

IT IS FURTHER ORDERED that all amounts withheld from the disposable earnings of KATHRYN COPELAND by the employer and paid in accordance with the order to that employer shall constitute a credit against the child support obligation. Payment of the full amount of child support ordered paid by this order through the means of withholding from earnings [REDACTED] charge



the child support obligation. If the amount withheld from earnings and credited against the child support obligation is less than 100 percent of the amount ordered to be paid by this order, the balance due remains an obligation of KATHRYN COPELAND, and it is hereby ORDERED that KATHRYN COPELAND pay the balance due directly to the state disbursement unit as specified below.

13.5 Order to Employer

On this date the Court authorized the issuance of an Income Withholding for Support.

13.6 Payment

IT IS ORDERED that all payments shall be made through the state disbursement unit at Texas Child Support Disbursement Unit, P.O. Box 659791, San Antonio, Texas 78265-9791, and thereafter promptly remitted to CHRISTOPHER J. COPELAND for the support of the children. IT IS ORDERED that all payments shall be made payable to the Office of the Attorney General and include the ten-digit Office of the Attorney General case number (if available), the cause number of this suit, KATHRYN COPELAND's name as the name of the noncustodial parent (NCP), and CHRISTOPHER J. COPELAND's name as the name of the custodial parent (CP).

Payment options are found on the Office of the Attorney General's website at www.texasattorneygeneral.gov/cs/payment-options-and-types.

IT IS ORDERED that each party shall pay, when due, all fees charged to that party by the state disbursement unit and any other agency statutorily authorized to charge a fee.

13.7 Change of Employment

IT IS FURTHER ORDERED that KATHRYN COPELAND shall notify this Court and CHRISTOPHER J. COPELAND by U.S. certified mail, return receipt requested, of any change of address and of any termination of employment. This notice shall be given no later than seven days after the change of address or the termination of employment. This notice or a subsequent notice shall also provide the current address of KATHRYN COPELAND and the name and address of her current employer, whenever that information becomes available.

13.8 Clerk's Duties

IT IS ORDERED that, on the request of a prosecuting attorney, the title IV-D agency, the friend of the Court, a domestic relations office, CHRISTOPHER J. COPELAND, KATHRYN COPELAND, or an attorney representing CHRISTOPHER J. COPELAND or KATHRYN COPELAND, the clerk of this Court shall cause a certified copy of the Income Withholding for Support to be delivered to any employer.

13.9 Notice

The Court may modify this order that provides for the support of a child,



- (1) the circumstances of the child or a person affected by the order have materially and substantially changed; or
- (2) it has been three years since the order was rendered or last modified and the monthly amount of the child support award under the order differs by either 20 percent or \$100 from the amount that would be awarded in accordance with the child support guidelines.

14. MEDICAL AND DENTAL SUPPORT

IT IS ORDERED that provided that the children qualify properly under KATHRYN COPELAND for Medicaid, then said Medicaid coverage shall be *the secondary coverage* for the children and both parents shall use it as such.

IT IS ORDERED that Christopher Copeland and Kathryn Copeland shall each provide medical support and dental support for the child as set out in this order as additional child support for as long as the Court may order Christopher Copeland and Kathryn Copeland to provide support for the child under sections 154.001 and 154.002 of the Texas Family Code. Beginning on the day Christopher Copeland and Kathryn Copeland's actual or potential obligation to support the child under sections 154.001 and 154.002 of the Family Code terminates, IT IS ORDERED that Christopher Copeland and Kathryn Copeland are discharged from the obligations set forth in this medical support order and dental support order with respect to that child, except for any failure by a parent to fully comply with those obligations before that date.

Definitions -

"Health insurance" means insurance coverage that provides basic health-care services, including usual physician services, office visits, hospitalization, and laboratory, X-ray, and emergency services, that may be provided through a health maintenance organization or other private or public organization, other than medical assistance under chapter 32 of the Texas Human Resources Code.

"Reasonable cost" of health insurance means the cost of health insurance coverage for a child that does not exceed 9 percent of Kathryn Copeland's annual resources, as described by section 154.062(b) of the Texas Family Code.

"Dental insurance" means insurance coverage that provides preventative dental care and other dental services, including usual dentist services, office visits, examinations, X-rays, and emergency services, that may be provided through a single service health maintenance organization or other private or public organization.



“Reasonable cost” of dental insurance means the cost of dental insurance coverage for a child that does not exceed 1.5 percent of Kathryn Copeland’s annual resources, as described by section 154.062(b) of the Texas Family Code.

“Health-care expenses” include, without limitation, medical, surgical, prescription drug, mental health-care services, dental, eye care, ophthalmological, and orthodontic charges but do not include expenses for travel to and from the provider or for nonprescription medication.

“Health-care expenses that are not reimbursed by insurance” (“unreimbursed expenses”) include related copayments and deductibles.

“Furnish” means -

- a. to hand deliver the document by a person eighteen years of age or older either to the recipient or to a person who is eighteen years of age or older and permanently resides with the recipient;
- b. to deliver the document to the recipient by first-class mail or by certified mail, return receipt requested, to the recipient’s last known mailing or residence address; or
- c. to deliver the document to the recipient at the recipient’s last known mailing or residence address using any person or entity whose principal business is that of a courier or deliverer of papers or documents either within or outside the United States.

Findings on Availability of Health Insurance—

Having considered the cost, accessibility, and quality of health insurance coverage available to the parties, the Court finds that health insurance is available or is in effect for the child through Christopher Copeland’s employment or membership in a union, trade association, or other organization at a reasonable cost.

IT IS FURTHER FOUND that the following orders regarding health-care coverage are in the best interest of the child.

Provision of Health-Care Coverage—

As additional child support, Christopher Copeland is ORDERED to maintain health insurance for the child as long as child support is payable for that child. Christopher Copeland is ORDERED—

- a. to furnish to each conservator of the child the following information no later than the thirtieth day after the date the notice of the rendition of this order is received:
 - i. Christopher Copeland’s Social Security number;



- ii. the name and address of Christopher Copeland's employer;
- iii. whether Christopher Copeland's employer is self-insured or has health insurance available;
- iv. proof that health insurance has been provided for the child;
- v. if Christopher Copeland's employer has health insurance available:
 - a. the name of the health insurance carrier;
 - b. the number of the policy;
 - c. a copy of the policy;
 - d. a schedule of benefits;
 - e. a health insurance membership card;
 - f. claim forms; and
 - g. any other information necessary to submit a claim; and
- vi. if Christopher Copeland's employer is self-insured:
 - a. a copy of the schedule of benefits;
 - b. a membership card;
 - c. claim forms; and
 - d. any other information necessary to submit a claim;
- b. to furnish to each conservator of the child a copy of any renewals or changes to the health insurance policy covering the child and any additional information regarding health insurance coverage of the child not later than the fifteenth day after Christopher Copeland receives or is provided with the renewal, change, or additional information;
- c. to notify each conservator of the child of any termination or lapse of the health insurance coverage of the child not later than the fifteenth day after the date of the termination or lapse;
- d. after a termination or lapse of health insurance coverage, to notify each conservator of the child of the availability to Christopher Copeland of additional health insurance for the child not later than the fifteenth day after the date the additional health insurance becomes available; and
- e. after a termination or lapse of health insurance coverage, to enroll the child in a health insurance plan that is available to Christopher Copeland at reasonable cost at the next available enrollment period.



Findings on Availability of Dental Insurance—

Having considered the cost, accessibility, and quality of dental insurance coverage available to the parties, the Court finds that dental insurance is available or is in effect for the child through Christopher Copeland's employment or membership in a union, trade association, or other organization at a reasonable cost.

IT IS FURTHER FOUND that the following orders regarding dental coverage are in the best interest of the child.

Provision of Dental Coverage—

As additional child support, Christopher Copeland is ORDERED to maintain dental insurance for the child as long as child support is payable for that child. Christopher Copeland is ORDERED—

- a. to furnish to each conservator of the child the following information no later than the thirtieth day after the date the notice of the rendition of this order is received:
 - i. Christopher Copeland's Social Security number;
 - ii. the name and address of Christopher Copeland's employer;
 - iii. whether Christopher Copeland's employer is self-insured or has dental insurance available;
 - iv. proof that dental insurance has been provided for the child;
 - v. if Christopher Copeland's employer has dental insurance available:
 - a. the name of the dental insurance carrier;
 - b. the number of the policy;
 - c. a copy of the policy;
 - d. a schedule of benefits;
 - e. a dental insurance membership card;
 - f. claim forms; and
 - g. any other information necessary to submit a claim; and
 - vi. if Christopher Copeland's employer is self-insured:
 - a. a copy of the schedule of benefits;
 - b. a membership card;
 - c. claim forms; and
 - d. any other information necessary to submit a claim;



- b. to furnish to each conservator of the child a copy of any renewals or changes to the dental insurance policy covering the child and any additional information regarding dental insurance coverage of the child not later than the fifteenth day after Christopher Copeland receives or is provided with the renewal, change, or additional information;
- c. to notify each conservator of the child of any termination or lapse of the dental insurance coverage of the child not later than the fifteenth day after the date of the termination or lapse;
- d. after a termination or lapse of dental insurance coverage, to notify each conservator of the child of the availability to Christopher Copeland of additional dental insurance for the child not later than the fifteenth day after the date the additional dental insurance becomes available; and
- e. after a termination or lapse of dental insurance coverage, to enroll the child in a dental insurance plan that is available to Christopher Copeland at reasonable cost at the next available enrollment period.

Medical Support Payments

Pursuant to section 154.182 of the Texas Family Code, IT IS ORDERED that KATHRYN COPELAND shall pay to CHRISTOPHER J. COPELAND cash medical support as additional child support, the amount of **Three Hundred Three and 00/100ths Dollars (\$303.00)** for medical, vision, and dental insurance costs with the first payment due and payable on June 1, 2024, and an identical payment of **Three Hundred Three and 00/100ths Dollars (\$303.00)** being due and payable on the first day of each month thereafter until both children have reached the age of 18 and graduated from high school.

IT IS FURTHER ORDERED that the **Income Withholding Order for Support** authorized above in this order shall include the payments for the cost of health insurance ordered herein.

IT IS FURTHER ORDERED that all payments for the cost of health insurance shall be made through the state disbursement unit at Texas Child Support Disbursement Unit, P.O. Box 659791, San Antonio, Texas 78265-9791. IT IS ORDERED that all payments shall be made payable to the Office of the Attorney General and include the ten-digit Office of the Attorney General case number (if available), the cause number of this suit, KATHRYN COPELAND's name as the name of the noncustodial parent (NCP), and CHRISTOPHER J. COPELAND's name as the name of the custodial parent (CP). Payment options are found on the Office of the Attorney General's website at www.texasattorneygeneral.gov/cs/payment-options-and-types.



IN THE SUPREME COURT OF TEXAS

No. _____

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals,
Fort Worth, Texas — No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court,
Tarrant County, Texas

APPENDIX

H

**Order on Supplemental Statement of Inability
to Afford Court Costs and Findings of Fact**

No. 231-686108-20, 231st Judicial District Court, Tarrant County, Texas
(Judicial finding: net resources effectively zero; disability prevents earning income)

Dated September 6, 2024

**NOTICE: THIS DOCUMENT
CONTAINS SENSITIVE DATA**

NO. 231-686108-20

IN THE INTEREST OF	§	IN THE DISTRICT COURT
	§	
Q.A.C. AND P.L.C.,	§	231st JUDICIAL DISTRICT
	§	
CHILDREN	§	TARRANT COUNTY, TEXAS

**ORDER ON SUPPLEMENTAL STATEMENT OF INABILITY TO
AFFORD COURT COSTS AND FINDINGS OF FACT**

On the 30th day of August 2024, the Court held a hearing on the appellant Kathryn Copeland's Statement of Inability to Afford Payment of Court Costs and any objections raised. This hearing was held in accordance with the Second Court of Appeals' Abatement Order dated August 6, 2024, which directed the trial court to conduct a hearing to review appellant Kathryn Copeland's Statement of Inability to Afford Court Costs and any objections thereto. Having reviewed the evidence presented, and pursuant to the Texas Rules of Civil Procedure and the Texas Rules of Appellate Procedure, the Court makes the following findings:

1. Material Change in Financial Circumstances:

- o The Court finds that Kathryn Copeland's financial circumstances have materially changed since the initial order on May 31, 2024. Specifically, she learned on August 12, 2024 that she has reached her lifetime maximum limit for federal unsubsidized loans. Thus, she no longer has that source of income to rely on for living expenses.

**ORDER ON SUPPLEMENTAL STATEMENT OF INABILITY TO
AFFORD COURT COSTS AND FINDINGS OF FACT**

1

- o The Court further finds that Copeland's neurologist, Dr. Victor Kayster, has confirmed that she is unable to work or participate in activities to prepare for work due to her disabilities, preventing her from earning an income.
- o The Court further finds that Kathryn Copeland received a letter denying her application for social security disability benefits on August 21, 2024, which she is in the process of appealing.

2. Income and Indigency Status:

- o The Court finds that Kathryn Copeland has no current source of income. She is reliant on government assistance, receiving \$291 per month in SNAP benefits and Medicaid for the Elderly and People with Disabilities.
- o The Court further finds that the appellant has demonstrated through testimony and evidence that her net resources are effectively zero, with no wages, earnings, or significant income sources.

3. Lack of Opposition Evidence:

- o The Court finds that appellee Christopher Copeland did not present any evidence or testimony at the hearing in opposition to Kathryn Copeland's Statement of Inability, and his initial objection did not comply with Texas Rule of Civil Procedure 145. 
- o The Court further finds that the objection filed by court reporter Shelley B. Mayo was withdrawn at the hearing.
- o The Court further finds that neither Christopher Copeland's nor Shelley B. Mayo's objections complied with Texas Rule of Civil Procedure 145, as both lacked sworn statements and personal knowledge of the facts. 

4. Findings of Indigency:

- o The Court finds that Kathryn Copeland has established her indigency by a preponderance of the evidence and is therefore entitled to proceed without paying court costs, including the costs for the clerk's record and reporter's record, pursuant to Texas Rules of Civil Procedure 145 and Texas Rules of Appellate Procedure 20.1.
- o The Court also finds that based on the appellant's financial circumstances, her net resources as claimed in her Statement of Inability to Afford Court Costs dated July 11, 2024, are substantiated and accurate.

ORDER

Based on the findings above, the Court ORDERS the following:

1. Kathryn Copeland is declared indigent and is entitled to the production of the clerk's record and reporter's record without the requirement of payment.
2. All objections to the appellant's Statement of Inability to Afford Court Costs are overruled, and appellant shall proceed without paying court costs.
3. The Clerk of the Court and the Court Reporter shall prepare and provide the necessary records for appellant's appeal in accordance with this order and applicable law.

SIGNED on September 6, 2024


JUDGE PRESIDING

ORDER ON SUPPLEMENTAL STATEMENT OF INABILITY TO
AFFORD COURT COSTS AND FINDINGS OF FACT

3

IN THE SUPREME COURT OF TEXAS

No. _____

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals,
Fort Worth, Texas — No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court,
Tarrant County, Texas

APPENDIX

I

**Per Curiam Order — Accepting Indigency Findings;
Appellant May Proceed Without Payment of Costs**

Second Court of Appeals, Fort Worth — No. 02-24-00278-CV

Dated September 17, 2024



**In the
Court of Appeals
Second Appellate District of Texas
at Fort Worth**

No. 02-24-00278-CV

IN THE INTEREST OF Q.C. AND P.C., CHILDREN

On Appeal from the 231st District Court
Tarrant County, Texas
Trial Court No. 231-686108-20

ORDER

On July 11, 2024, appellant filed an “Appellant’s Supplement to her Motion for Reconsideration and Reassessment of Indigency Determination,” claiming that her financial circumstances had materially changed since the trial court’s May 31, 2024 order requiring her to pay costs. *See* Tex. R. App. P. 20.1(b)(3). The appellee and the court reporter filed objections to appellant’s “Supplement,” so we abated the appeal and referred the motion to the trial court to issue findings of fact on appellant’s ability to afford court costs. *See id.*

The trial court’s supplemental record was filed in this court on September 9, 2024, which included the trial court’s order finding that appellant is now indigent and

is not required to pay costs. Accordingly, we grant appellant's motion claiming a material change in appellant's financial circumstances and likewise order that appellant shall proceed in this appeal without payment of costs.

The clerk and reporter's record are due **September 30, 2024**.

We direct the clerk of this court to send a notice of this order to the appellant, all lead counsel on appeal, the trial court judge, the trial court clerk, and the court reporter.

Dated September 17, 2024.

Per Curiam

IN THE SUPREME COURT OF TEXAS

No. _____

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals,
Fort Worth, Texas — No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court,
Tarrant County, Texas

APPENDIX

J

**5th Circuit Order Denying Reconsideration of Motion for Injunction
Pending Appeal — *Copeland v. Tarrant County***

Judge Catharina Haynes noted she would grant relief consistent with her
prior footnote on May 6, 2026.

Federal Fifth Circuit Court of Appeals, New Orleans — No. 26-10389

Dated May 20, 2026

United States Court of Appeals

FIFTH CIRCUIT
OFFICE OF THE CLERK

LYLE W. CAYCE
CLERK

TEL. 504-310-7700
600 S. MAESTRI PLACE,
Suite 115
NEW ORLEANS, LA 70130

May 20, 2026

MEMORANDUM TO COUNSEL OR PARTIES LISTED BELOW:

No. 26-10389 Copeland v. Tarrant County
USDC No. 4:25-CV-890

Enclosed is an order entered in this case.

Sincerely,

LYLE W. CAYCE, Clerk

Lisa E. Ferrara

By:

Lisa E. Ferrara, Deputy Clerk
504-310-7675

Ms. Kathryn Copeland
Mr. Stephen Andrew Lund
Ms. Katherine E. Owens

United States Court of Appeals
for the Fifth Circuit

United States Court of Appeals
Fifth Circuit

FILED

May 20, 2026

Lyle W. Cayce
Clerk

No. 26-10389

KATHRYN COPELAND,

Plaintiff—Appellant,

versus

TARRANT COUNTY, TEXAS; OFFICE OF COURT
ADMINISTRATION; JUDITH WELLS; KENNETH NEWELL; DAVID
EVANS,

Defendants—Appellees.

Appeal from the United States District Court
for the Northern District of Texas
USDC No. 4:25-CV-890

UNPUBLISHED ORDER

Before SMITH, HAYNES, and OLDHAM, *Circuit Judges*.

PER CURIAM:

This panel previously DENIED Appellant's motion for injunction pending appeal. The panel has considered Appellant's motion for reconsideration.

IT IS ORDERED that the motion is DENIED.¹

¹ Judge Haynes would grant it along the lines of her prior footnote.

United States Court of Appeals
for the Fifth Circuit

No. 26-10389

United States Court of Appeals
Fifth Circuit

FILED

May 6, 2026

Lyle W. Cayce
Clerk

KATHRYN COPELAND,

Plaintiff—Appellant,

versus

TARRANT COUNTY, TEXAS; OFFICE OF COURT
ADMINISTRATION; JUDITH WELLS; KENNETH NEWELL; DAVID
EVANS,

Defendants—Appellees.

Appeal from the United States District Court
for the Northern District of Texas
USDC No. 4:25-CV-890

UNPUBLISHED ORDER

Before SMITH, HAYNES, and OLDHAM, *Circuit Judges*.

PER CURIAM:

IT IS ORDERED Appellant's opposed motion for injunction pending appeal is DENIED.¹

¹ Judges Haynes would grant temporarily and carry with the case for the merits panel to decide whether to maintain the injunction pending appeal.

United States Court of Appeals

FIFTH CIRCUIT
OFFICE OF THE CLERK

LYLE W. CAYCE
CLERK

TEL. 504-310-7700
600 S. MAESTRI PLACE,
Suite 115
NEW ORLEANS, LA 70130

May 06, 2026

MEMORANDUM TO COUNSEL OR PARTIES LISTED BELOW:

No. 26-10389 Copeland v. Tarrant County
USDC No. 4:25-CV-890

Enclosed is an order entered in this case.

Sincerely,

LYLE W. CAYCE, Clerk



By: Melissa B. Courseault, Deputy Clerk
504-310-7701

Ms. Kathryn Copeland
Mr. Stephen Andrew Lund
Ms. Karen S. Mitchell
Ms. Katherine E. Owens

IN THE SUPREME COURT OF TEXAS

No. _____

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals,
Fort Worth, Texas — No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court,
Tarrant County, Texas

APPENDIX

K

**Petitioner's Request for Permission to File Emergency Motion for
Temporary Orders.**

Documents Q.C.'s medical emergency and exhaustion of § 11.102 prefiling
restriction remedies.

330th District Court, Dallas — No. DF-25-02212

Filed and Accepted — May 4, 2026

**NOTICE: THIS DOCUMENT
CONTAINS SENSITIVE DATA**

NO. DF-25-02212

IN THE INTEREST OF	§	IN THE DISTRICT COURT
	§	
Q.A.C. AND P.L.C.,	§	330TH JUDICIAL DISTRICT
	§	
CHILDREN	§	DALLAS COUNTY, TEXAS

**Petitioner’s Request for Permission to File An Emergency Motion for
Temporary Orders**

TO THE HONORABLE ANDREA D. PLUMLEE,

LOCAL ADMINISTRATIVE JUDGE:

This filing comes with a prefiling restriction. This Court is therefore the first reader — and the gatekeeper of whether what follows ever reaches the docket.

What this looks like: a litigant under a § 11.102 order, filing again.

What this is: a mother holding a referral slip.

In November 2024 — months after the final custody order — [REDACTED] own pediatrician documented a diagnosis of Avoidant/Restrictive Food Intake Disorder and handed [REDACTED] father the referral information for Children’s Health Eating Disorders Clinic. (App. 02-07.) His written position today, eighteen months later:

“She doesn’t have ARFID. There is nothing to discuss.” — OFW Message, April 30, 2026 (App. 23.)

Kathryn Copeland is a licensed Texas attorney appearing pro se. She is also a woman with a diagnosed genetic connective tissue disorder — the same class of condition presenting in her daughters’ documented medical records. (App. 22.) She does not raise this for sympathy. She raises it because when [REDACTED] pediatric cardiologist documented a resting heart rate of 113 BPM at the January 2026 cardiology visit (App. 01), when [REDACTED] height has not increased by a single measurable centimeter in four months, when [REDACTED] has had no eyebrows or eyelashes for three consecutive years under her father’s full custody — Kathryn Copeland is not guessing at what that means. She knows.

She has asked. She has documented. She has waited. On May 3, 2026, she made one more written request for a multidisciplinary evaluation. The response:

“I have already addressed this with you multiple times. My previous correspondence on this matter stands.” — OFW Message, May 3, 2026.

[REDACTED] Copeland is fourteen years and three months old. She is finishing eighth grade. The growth window that closes in late adolescence does not reopen. (App. 14-20.)

This Court has the authority to let this matter proceed. The evidence that follows demonstrates — objectively, with medical records and written communications — that there is good cause to do so.

There is no risk of harm in granting this permission. There is predictable, preventable harm in declining it.

I. THE NARROW ISSUE

Applicant seeks permission to file a single, narrow Emergency Motion for Temporary Orders. The motion seeks temporary authority to obtain a multidisciplinary medical evaluation for Q.A.C. (██████) age 14, who has a documented diagnosis of Avoidant/Restrictive Food Intake Disorder (ARFID), documented growth failure, and a documented physician referral for specialized treatment that Respondent has refused to pursue for eighteen months. (App. 01-12.)

The issue before this Court is not a disagreement between parents. It is whether necessary medical evaluation can proceed where one parent has declined such evaluation despite a physician's referral, objective clinical indicators, and three written refusals within a five-day period. Delay during a time-limited developmental window creates foreseeable, irreversible harm.

II. BACKGROUND AND CHANGED CIRCUMSTANCES

A. Documented Diagnosis and Prior Referral (App. 02–05)

On November 13, 2024, ██████ was evaluated by Tammi L. Schlichtemeier, MD at Coppell Pediatric Associates. Dr. Schlichtemeier documented the following active diagnoses:

- **ARFID (F50.82)** — Avoidant/Restrictive Food Intake Disorder, date diagnosis added to chart September 10, 2024 (App. 03)
- **Trichotillomania (F63.3)** — [REDACTED] has been pulling hair from her body and head, including eyebrows and eyelashes, for an extended period (App. 03, App. 05)
- **Generalized Anxiety Disorder (F41.1)** — Active, despite father's claim that counseling "ruled out anxiety as a cause" (App. 03, App. 05)
- **Pure Hypercholesterolemia** — Active, date discovered September 10, 2024 (App. 03)
- **BMI: 5th–85th percentile for age** (App. 03)

"Gave father referral info for Children's Health Eating Disorders Clinic. Difficult to increase wt gain due to ARFID." (App. 05.)

The father was given the referral directly at that appointment. It has never been pursued.

"Father states that through counseling 'we have ruled out anxiety as a cause.' Yet she has a diagnosis of Generalized Anxiety Disorder from psychiatrist." (App. 05.)

[REDACTED] vitals at the November 13, 2024 visit: Height 141.6 cm, Weight 32.57 kg, BMI 15.25th percentile, Heart Rate 102 BPM (App. 04).

P.L.C. ([REDACTED] age 10, presents with weight at the 3rd percentile and height at the 15th percentile as of May 4, 2026, with early evidence of downward percentile crossing. (App. 13.)

B. Objective Medical Indicators: Growth Failure and Tachycardia

Cardiology Visit — January 29, 2026 (App. 01): [REDACTED] was seen at Pediatric Heart Specialists – DFW by Dr. Merick Yamada, MD for screening for heart disease and high cholesterol. Documented vitals:

- Pulse: 113 BPM (elevated resting tachycardia for a 13-year-old)
- Height: 59 inches — 6th percentile (down from prior measurements)
- Weight: 92 lb 12.8 oz — 19th percentile
- Blood Pressure: 104/58
- BMI: 18.74 — 42nd percentile
- Labs ordered: Lipid panel; follow-up scheduled July 29, 2026 for cholesterol review

Elevated cholesterol in an adolescent with growth restriction and nutritional concerns may indicate metabolic compensation for nutritional deficit — a clinical pattern associated with restrictive nutritional intake.

Growth Failure — January to May 2026 (App. 08-09): [REDACTED] height measurements from January 5, 2026 through May 4, 2026 show zero measurable growth over four months during early adolescence — a critical developmental window. Height lost during this period cannot be recovered once growth plates close.

C. Explicit Refusal of Evaluation (App. 23-25)

Applicant made multiple written attempts to obtain Respondent's agreement to pursue the physician-referenced evaluation. Respondent's written responses:

- **April 30, 2026, 10:47 AM:** “[REDACTED] has been thriving. ... There is nothing to discuss.” (App. 24.)
- **April 30, 2026, 1:24 PM:** “She doesn’t have ARFID.” (App. 23.)
- **May 3, 2026, 3:08 PM:** “I have already addressed this with you multiple times. My previous correspondence on this matter stands.”

D. Pending Challenge to the Vexatious Litigant Order

Applicant advises this Court that the January 21, 2025 vexatious litigant order creating the prefiling requirement is the subject of pending legal challenges:

- On April 19, 2026, Applicant filed in this Court (DF-25-02212) a Motion to Declare the January 21, 2025 Vexatious Litigant Order Void Ab Initio on five independent grounds, including: (1) lack of subject matter jurisdiction — the transferor court’s plenary power expired June 30, 2024, making the November 8, 2024 motion and January 21, 2025 order both untimely by more than 200 days; (2) mandatory transfer having divested the court of all authority; and (3) evidentiary failure under *Serafine v. Crump*, 691 S.W.3d 917 (Tex. 2024), as the court itself acknowledged the supporting argument was “one of legalities and not on evidentiary evidence.” That motion is pending in this Court without a ruling.
- Applicant has also filed a challenge with the Dallas Court of Appeals and has not received a response.

Granting permission under § 11.102 is appropriate regardless of how those challenges resolve. The children’s medical needs do not pause for interlocutory proceedings.

E. Prior Rejected Filings — No Alternative Remedy Available

On April 19, 2026, Applicant attempted to file in this Court a Verified Petition for Bill of Review, a supporting Brief, a Verified Affidavit, and a Petition for Declaratory Judgment. Those filings were rejected by the clerk under the § 11.102 prefiling restriction. Applicant has therefore exhausted all available non-judicial channels. There is no alternative path to obtaining medical relief for [REDACTED] without this Court’s permission.

III. IRREPARABLE HARM AND TIME SENSITIVITY

The harm is time-sensitive and, in critical respects, **irreversible** (App. 14-17):

- **Height stunting is permanent.** Once growth plates close in late adolescence, height lost during the critical early adolescent window cannot be recovered by any future order or intervention. The four-month height flatline represents unreversible loss of growth potential.
- **Nutritional deficit during adolescence affects bone density, cognitive development, and immune function** (App. 17) — all with long-term, compounding consequences. (App. 14.)

- **ARFID untreated for 18 months typically becomes more entrenched.** The November 2024 eating disorders program referral represents the treatment window most likely to result in full recovery. That window is closing.
- **[REDACTED] is 14 years and 3 months old, finishing 8th grade.** The summer before high school is among the most clinically significant intervention windows available. After this summer, the social and developmental complexity of high school makes treatment substantially harder to initiate and sustain.

IV. MERITORIOUSNESS OF THE PROPOSED FILING

A. The Litigation Has Merit and Is Not Frivolous

The proposed filing is supported by objective documentary evidence:

- Active ARFID diagnosis (F50.82), documented by treating pediatrician, September 10, 2024 (App. 03)
- Physician-generated referral to Children’s Health Eating Disorders Clinic, given to Respondent November 13, 2024 (App. 05)
- Resting heart rate of 113 BPM documented at cardiology visit, January 29, 2026 (App. 01)
- Height at 6th percentile with zero measurable growth over four months during early adolescence (App. 01; 08)

- Three written refusals by Respondent within a five-day period, including written denial that the ARFID diagnosis exists (App. 23-25)

The requested relief is narrow and limited to temporary medical decision-making authority sufficient to permit evaluation. It is not a collateral attack on the custody order.

B. Immediate Danger of Irreparable Harm

- Zero centimeters of measurable growth in four months during early adolescence (App. 08-09)
- Resting heart rate of 113 BPM requiring clinical investigation (App. 01)
- Age 14, 8th grade, critical growth and treatment window closing before high school begins
- Eighteen months of documented, unremedied clinical indicators since ARFID diagnosis

C. No Adequate Alternative Remedy

Respondent has stated in writing that there is “nothing to discuss.”  cannot self-advocate for medical care. Applicant’s April 19, 2026 broader filings were rejected by the clerk. The § 11.102 permission request is the only available path.

V. REQUEST FOR ADA ACCOMMODATION

Applicant respectfully requests that this matter be decided on written submission without oral hearing, pursuant to Title II of the Americans with Disabilities Act, 42 U.S.C. § 12101 et seq. Applicant has a documented brain injury and autonomic dysfunction affecting real-time verbal processing. Written briefing format is sufficient to resolve this request and does not prejudice any party.

VI. PROPOSED RELIEF

Applicant seeks permission to file an Emergency Motion for Temporary Orders requesting, in the alternative:

- Temporary authority for Applicant to obtain multidisciplinary evaluation for [REDACTED] documented ARFID diagnosis, growth concerns, and cardiovascular findings; OR
- An order requiring Respondent to proceed with a multidisciplinary eating disorders evaluation with Children's Health Eating Disorders Clinic (or equivalent specialist facility) within 14 days of the order; OR
- Transfer of temporary medical decision-making authority to Applicant for evaluation and evidence-based treatment of the documented feeding disorder, growth concerns, and related conditions.

This relief is limited and temporary, designed to permit evidence-based medical evaluation while the Court can consider the matter on a fuller record.

VII. CONCLUSION AND PRAYER

For thirty-three months, Applicant has attempted to resolve these escalating medical concerns through every available out-of-court channel, only to be met with a persistent denial of clinical reality. The biological data — the height flatline, the abnormal cardiovascular vitals, the active diagnosis with no treatment — confirm that the window for effective intervention is closing.

Applicant is not asking this Court to decide final conservatorship. She is asking for permission to present objective medical evidence and to request temporary orders necessary to protect the child's health while the Court can consider the matter on a fuller record.

There is no risk of harm in granting this permission. There is predictable, preventable harm in declining it.

Applicant respectfully prays that the Local Administrative Judge:

- Grant permission to file the Emergency Motion for Temporary Orders; and
- Direct that the matter be decided on written submission, consistent with Applicant's ADA accommodation request.

Respectfully Submitted,



KATHRYN COPELAND

Pro Se (Licensed Attorney)

State Bar No. 24086056

1137 S. Pine Street

Grapevine, Texas 76051

Katie@CopelandLawTexas.com

Petitioner, Pro Se

**UNSWORN DECLARATION PURSUANT TO
TEX. CIV. PRAC. & REM. CODE 132.001**

“My name is Kathryn Marie Copeland, my date of birth is June 28, 1985 and my current address is 1137 S. Pine St., Grapevine, Texas 76051 in the United States of America. I declare under penalty of perjury that the foregoing is true and correct. Executed in Tarrant County, Texas on May 4, 2026.”



Kathryn Copeland

PETITIONER

VERIFICATION OF EXHIBITS

I, KATHRYN MARIE COPELAND, declare under penalty of perjury pursuant to Texas Civil Practice and Remedies Code § 132.001 that the foregoing is true and correct to the best of my knowledge and belief:

1. I am the Movant Mother in this cause and have personal knowledge of the facts stated;
2. The exhibits attached are true and correct copies of documents I have reviewed or received;
3. The medical measurements and dates are accurate to the best of my knowledge;
4. The OurFamilyWizard messages are accurate transcripts of written communications; and
5. The statements attributed to Respondent are direct quotations from those communications.



Kathryn Copeland

PETITIONER

CERTIFICATE OF CONFERENCE

I, Kathryn Copeland, the undersigned attorney pro se, hereby certify to the Court that:

Movant and Respondent have personally conducted a conference on May 3, 2026 via OurFamilyWizard at which there was a substantive discussion of every item presented to the Court in this motion and despite best efforts the counsel have not been able to resolve those matters presented.

Certified to the 4th day of May, 2026 by Kathryn Copeland



Kathryn Copeland

CERTIFICATE OF EMERGENCY (DALLAS LOCAL RULE 2.07)

An emergency exists of such a nature that further delay would cause irreparable harm to the movant and children, as follows:

Q.A.C. ([REDACTED]) age 14, has a documented diagnosis of Avoidant/Restrictive Food Intake Disorder (ARFID) since September 2024. Her treating pediatrician provided Respondent with a referral to Children's Health Eating Disorders Clinic in November 2024. Respondent has refused to pursue that evaluation and stated in writing on May 3, 2026 that [REDACTED] "doesn't have ARFID" and there is "nothing to discuss."

[REDACTED] January 29, 2026 cardiology evaluation documented a resting heart rate of 113 BPM and height at the 6th percentile. Her height has not increased by a single measurable centimeter from January 5 through May 4, 2026 — a four-month growth flatline during a critical developmental window. Height stunting during early adolescence is irreversible once growth plates close. [REDACTED] is finishing 8th grade; the summer before high school is among the most effective treatment windows available. Further delay forecloses it permanently.

Applicant's April 19, 2026 filings seeking related relief were rejected under the prefiling restriction. There is no adequate alternative remedy.


KATHRYN COPELAND

CERTIFICATE OF SERVICE

I certify that a true and correct copy of the above and foregoing document was served on all counsel of record in accordance with the Texas Rules of Civil Procedure on May 4, 2026.


KATHRYN COPELAND

EXHIBITS LIST WITH BATES REFERENCES

January 29, 2026 Cardiology After Visit Summary (Bates: App. 01)

Pediatric Heart Specialists – DFW, Dr. Merick Yamada, MD

- Pulse: 113 BPM; Height: 59” (6th percentile); Weight: 92 lb 12.8 oz (19th percentile)
- Lipid panel ordered; follow-up July 2026 for cholesterol review

November 13, 2024 Pediatric Visit Summary (Bates: App. 02–07)

Coppell Pediatric Associates, Dr. Tammi L. Schlichtemeier, MD

- *App. 02:* Clinical Summary cover — Guardian listed as Christopher Copeland; encounter date November 13, 2024
- *App. 03:* Problems list — ARFID (F50.82) active since 09/10/2024; Trichotillomania (F63.3); GAD (F41.1); Hypercholesterolemia; BMI 5th–85th %ile
- *App. 04:* Vitals 11/13/2024 — Height 141.6 cm, Weight 32.57 kg, BMI 15.25th %ile, HR 102 BPM; Assessment confirms F50.82, F63.3, F41.1
- *App. 05:* Plan of Treatment — ARFID: “Gave father referral info for Children’s Health Eating Disorders Clinic.” Trichotillomania: father’s erroneous denial of anxiety documented against active GAD diagnosis

Growth Data: Height Measurements through April 6, 2026 (Bates: App. 08-10)

[REDACTED] Growth Chart — *Key finding:* Zero measurable height growth in four months during early adolescence

Treatment Program for ARFID (Bates: App. 11-12)

May 4, 2026 [REDACTED] Sick Visit (Bates: App. 13)

Weight: 3rd percentile; BMI: 4th percentile; *Purpose:* Establishes pattern of health concerns across both children under Respondent’s primary custody.

Journal of Eating Disorders; *Unique considerations for the medical care of restrictive eating disorders in children and young adolescents* by Anna B. Tanner (2023) (Bates: App. 14-20)

<https://doi.org/10.1186/s40337-023-00759-2>

January 5, 2026 [REDACTED] Lab Work (Bates: App. 21)

November 9, 2023 Referral for Hypermobility Joints (Bates: App. 22)

OurFamilyWizard Messages, April 30 – May 3, 2026 (Bates: App. 23-25)

- *April 30, 10:47 AM:* “[REDACTED] has been thriving. ... There is nothing to discuss.”
- *April 30, 1:24 PM:* “She doesn’t have ARFID.”

Name: [REDACTED] Copeland | DOB: [REDACTED] 2012 | MRN: 5025690 | PCP: Tammi L Schlichtemeier, MD | Legal Name: [REDACTED]
[REDACTED] Copeland

Office Visit - Jan 29, 2026 ([REDACTED])

with Merick Yamada, MD at Pediatric Heart Specialists - DFW

After Visit Summary®

AFTER VISIT SUMMARY

[REDACTED] Copeland MRN: 5025690
CEID: DCM-L8NB-33TS-5RD3

1/29/2026 1:30 PM Pediatric Heart
Specialists - DFW 972-331-9700

Instructions from Merick Yamada, MD



Labs ordered today
OSH-LIPID PANEL W/ NON-HDL & CHOL/HDL
Complete as directed by your provider.



Return in about 6 months
(around 7/29/2026) for lipid panel review.

Today's Visit



You saw Merick Yamada, MD on Thursday January 29, 2026. The following issues were addressed: Screening for heart disease and High cholesterol.



Weight
92 lb 12.8 oz (19th percentile)



Height
59" (6th percentile)



Blood Pressure
104/58



BMI
18.74 (42nd percentile)



Pulse
113



Respiration
20



Oxygen Saturation
98%

Clinical Summary

Patient	████ COPELAND
Date of birth	████ 2012
Sex	Female
Race	White
Ethnicity	Not Hispanic or Latino
Language	en Language
Contact info	Primary Home: 1331 VALLEY VISTA DRIVE Irving, TX 75063, US Tel: +1(817)789-9782
Patient IDs	COPQUI0001 2.16.840.1.113883.4.317
Document Id	a3d26992-064d-4cc4-84cb-d563b5bf574f
Document Created:	November 17, 2024, 19:44:16
Performer	TAMMI SCHLICHTEMEIER, MD of COPPELL PEDIATRIC ASSOCIATES, PA
Performer	JOVANNY ESTRADA, MA of COPPELL PEDIATRIC ASSOCIATES, PA
Author	TAMMI SCHLICHTEMEIER, MD
Contact info	Work Place: 1705 EAST BELTINE ROAD Coppell, TX 75019-9606, US Tel: +1(972)393-8687
Author	eMDs;DocumentBuilder:2.7.3.2:2493
Contact info	Work Place: 1705 EAST BELTLINE RD Coppell, TX 75019-9606, US Tel: +1(972)393-8687
Encounter Id	cf6ee73c-9ad7-44cd-adae-0448427d69d0
Encounter Date	From November 13, 2024, 09:09:00
Encounter Location	
Responsible party	TAMMI SCHLICHTEMEIER, MD
Guardian	CHRISTOPHER COPELAND
Contact info	Primary Home: 1331 VALLEY VISTA DRIVE Irving, TX 75063- , US Tel: +1(817)789-9782
Legal authenticator	TAMMI SCHLICHTEMEIER, MD signed at November 17, 2024, 19:44:16
Contact info	Work Place: 1705 EAST BELTINE ROAD Coppell, TX 75019-9606, US Tel: +1(972)393-8687
Document maintained by	COPPELL PEDIATRIC ASSOCIATES, PA
Contact info	Work Place: 1705 EAST BELTLINE RD Coppell, TX 75019-9606, US Tel: +1(972)393-8687

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Social History.

Code Name	Start Date	Stop Date
Never smoker	09/28/2024	
Never Smoker	07/07/2024	
Never smoker	07/03/2024	

Birth Sex: Female

Problems

SNOMED	Problem	Status	Date Discovered	Last Modified Date
	Avoidant/restrictive food intake disorder	Active	09/10/2024	11/17/2024
238958004	Excoriation (skin-picking) disorder	Active	07/07/2024	11/17/2024
	Major depressive disorder, single episode, in full remission	Active	07/07/2024	11/17/2024
	Generalized anxiety disorder	Active	07/07/2024	11/17/2024
	Trichotillomania	Active	07/07/2024	11/17/2024
	Epigastric pain	Active	11/13/2024	11/17/2024
	Body mass index (BMI) pediatric, 5th percentile to less than 85th percentile for age	Active	11/13/2024	11/17/2024
	Pure hypercholesterolemia, unspecified	Active	09/10/2024	09/10/2024
	Disruption of family by separation and divorce	Active	07/07/2024	07/07/2024
	Major depressive disorder, single episode, moderate	Active	07/07/2024	07/07/2024

Medications

Brand	Strength	Dose/Route/Frequency	RxNorm Code	Date Started	Date Discontinued	Status	Last Modified Date
sertraline	100 mg oral tablet	TAKE 1 TABLET BY MOUTH EVERY DAY	312938	06/20/2024		Current	11/13/2024
sertraline	25 mg oral tablet	TAKE 1 TABLET BY MOUTH EVERY DAY	312940	05/31/2024		Current	11/13/2024
atomoxetine	18 mg oral capsule	TAKE 1 CAPSULE BY MOUTH EVERY DAY IN THE MORNING	349592	05/10/2024		Current	11/13/2024
omeprazole	20 mg oral capsule, delayed release (enteric coated)	1 CAP PO Q DAY 30 MIN BEFORE DINNER ON EMPTY STOMACH.	198051	11/13/2024		Current	11/13/2024

Allergies

No allergies listed.

Procedures

Code System	Code	Description	Date Ordered	Status
CPT	3008F	Body Mass Index (BMI), documented (PV)	11/13/2024	completed

Immunizations

Date	Vaccine	Status	CVX	Last Modified Date
12/05/2022	COVID-19, mRNA, LNP-S, bivalent, PF, 10 mcg/0.2 mL dose	Completed	301	07/03/2024

04/27/2012	DTaP, unspecified formulation	Completed	107	07/02/2024
06/26/2012	DTaP, unspecified formulation	Completed	107	07/02/2024
05/28/2013	DTaP, unspecified formulation	Completed	107	07/02/2024
02/25/2013	Hep A, ped/adol, 2 dose	Completed	83	07/03/2024
12/19/2019	Hep A, ped/adol, 2 dose	Completed	83	07/03/2024
2012	Hep B, adolescent or pediatric	Completed	8	07/03/2024
03/26/2012	Hep B, adolescent or pediatric	Completed	8	07/03/2024
11/27/2012	Hep B, adolescent or pediatric	Completed	8	07/03/2024
04/27/2012	Hib, unspecified formulation	Completed	17	07/03/2024
06/26/2012	Hib, unspecified formulation	Completed	17	07/03/2024
08/27/2012	Hib, unspecified formulation	Completed	17	07/03/2024
05/28/2013	Hib, unspecified formulation	Completed	17	07/03/2024
07/03/2024	HPV9 (GARDASIL 9)	Completed	165	07/03/2024
09/28/2024	Influenza, split virus, trivalent, PF (FLUZONE TRIVALENT 2024-2025)	Completed	140	09/28/2024
08/27/2012	influenza, unspecified formulation	Completed	88	07/03/2024
11/27/2012	influenza, unspecified formulation	Completed	88	07/03/2024
12/05/2017	influenza, unspecified formulation	Completed	88	07/03/2024
12/19/2019	influenza, unspecified formulation	Completed	88	07/03/2024
03/25/2021	influenza, unspecified formulation	Completed	88	07/03/2024
04/27/2012	IPV	Completed	10	07/03/2024
06/26/2012	IPV	Completed	10	07/03/2024
08/27/2012	IPV	Completed	10	07/03/2024
06/14/2023	meningococcal MCV4, unspecified formulation	Completed	147	07/03/2024
02/25/2013	MMR	Completed	3	07/03/2024
06/14/2023	Tdap	Completed	115	07/03/2024
01/11/2022	typhoid, unspecified formulation	Completed	91	07/03/2024
02/25/2013	varicella	Completed	21	07/03/2024
08/27/2012	DTaP	Completed	20	07/02/2024
12/19/2019	MMRV	Completed	94	07/03/2024
12/19/2019	DTaP-IPV	Completed	130	07/02/2024
06/14/2023	HPV9	Completed	165	07/03/2024

Vital Signs

Date	PulseOx	Height	Weight	BMI	BMI %	Temp	Respiration	Heart Rate	Blood Pressure	IO2C	Wt-Ht %	HC	HC %
11/13/2024	99%	141.6cm	32.57kg	16.2kg/m2	15.25%	36.89Cel	26/min	102/min	121/76mm[Hg]	21%			

Assessment

- R10.13 Epigastric pain
- F63.3 Trichotillomania
- F32.5 Major depressive disorder, single episode, in full remission
- F41.1 Generalized anxiety disorder
- F42.4 Excoriation (skin-picking) disorder
- F50.82 Avoidant/restrictive food intake disorder
- Z68.52 Body mass index (BMI) pediatric, 5th percentile to less than 85th percentile for age

Plan of Treatment

- Epigastric pain
 - . 1. Considering psych diagnoses and location of pain/ discomfort persistent, I cannot rule out gastric or duodenal ulcer, but do suspect GERD. 2. Rec trial of Omeprazole 20mg po q day x 2 wks. 3. If no improvement, plans to refer to GI Dr John Baker for possible endoscopy and H pylori breath test. 4. Call MD if increasing pain or increasing frequency. 5. RTC 2 wk recheck and pm. 6. Diet: No caffeine, carbonated drinks, chocolate, acidic foods, tomato, tomato sauce, cucumbers, pickles, garlic, MSG or spicy foods.
 - Prescriptions: :
[New Rx] omeprazole 20 mg oral capsule, delayed release (enteric coated) [1 CAP PO Q DAY 30 MIN BEFORE DINNER ON EMPTY STOMACH.], #30 (thirty) capsules, Refills: 1 (one)
 - Orders: :
99214 - Established patient outpatient visit, Moderate MDM and/or 30-39 minutes (33 minutes)
- Trichotillomania
 - . 1. At some point, pt had GAD, which may well have played a role in her trichotillomania onset. 2. Now she may have sx due to anxiety or due to habit or environmental stressors. 3. Suspect GAD may play role in possible gastritis currently. 4. Continue Sertraline 100mg po qhs.
- Major depressive disorder, single episode, in full remission
 - . 1. No sx of depression in over 1 year per pt and dad. 2. Will consider change to Escitalopram for coverage of anxiety and see if trichotillomania improves and if no, then may wean to see if any depressive or GAD sx recur. ts
- Generalized anxiety disorder
- Excoriation (skin-picking) disorder
- Avoidant/restrictive food intake disorder
 - . Gave father referral info for Childrens Health Eating Disorders Clinic. 2. Difficult to increase wt gain due to ARFID. ts
- Body mass index (BMI) pediatric, 5th percentile to less than 85th percentile for age
 - Orders: :
3008F - Body Mass Index (BMI), documented (PV)

Referrals

No referral reasons listed.

Functional Status

No Functional Status Listed

Mental Status

No Mental Status Listed

Goals

No Goals listed.

Health Concerns

No Health Concerns listed.

Health Status Evaluations/Outcomes

No Evaluations/Outcomes listed.

Interventions

No Interventions listed.

Lab Results

No lab results listed.

Encounter Diagnosis

- Generalized abdominal pain

Generalized abdominal pain noted. It began 6 days ago. It is localized primarily in the epigastric area and periumbilical area. It does not seem to radiate. She characterizes the pain as cramping and sharp. It is typically of moderate intensity. The discomfort occurs, on average, "comes in waves".. The duration is extremely variable. Associated symptoms include sour taste in mouth. She has not experienced abdominal distention or bloating, anorexia, constipation, dysuria, fever, myalgia, shortness of breath or vomiting. Nothing helps alleviate the pain. There are no known aggravating factors. There are no obvious stressors in her life. There is no evidence of school phobia. No family members or other contacts are similarly ill. denies recent travel.

Pt has constipation x several days and dad had been giving her N Acetyl Cysteine 600mg po q day and stopped 3 days ago, b/c read that NAC can cause constipation. Pt notes BM q day x 3 days and Bristol Type 4 or 5. No hard or large diam stools. No pain on defecation. No bld in or on stools. ts
- Nausea
- Trichotillomania

c/o pt continues to bite nails frequently as well as pulling out eyebrows frequently. Father states that through counseling "we have ruled out anxiety as a cause". Yet she has a diagnosis of Generalized Anxiety Disorder from psychiatrist. Pt on Sertraline for > 1 year at 100mg but no change in dose with incr in sx of trichotillomania. Pt denies eating hair from body or head except eyebrows. Pt does eat fingernails and toenails and peeled dead skin from body. Denies eating paper, toilet or tissue paper, polyfil, stuffing, etc.
- Major depressive disorder, single episode, moderate

Dad notes that psychiatrist has retired and wants to have PCP manage meds. He does have appt at DFW Child Psychiatry in 1 month if PCP chooses not to manage meds. Pt has counselor appts x 1 mo. with someone who "also has trichotillomania" and "specializes in it". ts

- Generalized anxiety disorder
- Excoriation (skin-picking) disorder
- Avoidant/restrictive food intake disorder

Has not started Feeding Therapy or had evaluation by Childrens Health Eating Disorders Clinic Dad states mom had info but did not make appt He requests referral info to make appt himself. ts

- Body mass index (BMI) pediatric, 5th percentile to less than 85th percentile for age

Progress

- Doctor Note:
COPELAND, [REDACTED] 2012
Default Progress Note
Visit Date: Wed, Nov 13, 2024 9:09 am
Provider: SCHLICHTEMEIER, TAMMI LYN, MD (Supervisor: SCHLICHTEMEIER, TAMMI LYN, MD; Assistant: ESTRADA, JOVANNY, MA)
Location: COPPELL PEDIATRIC ASSOCIATES, PA

Electronically signed by TAMMI SCHLICHTEMEIER, MD on 11/17/2024 07:44:03 PM
 Electronically signed by SCHLICHTEMEIER, TAMMI LYN, MD on 11/17/24 7:44:04 pm

Subjective:

CC: [REDACTED] is a 12 year old White female. She presents with abdominal pain and nausea. BODY MASS INDEX-- 5 TO 84% The history is obtained from the patient, the patient's mother, and due to patient inability to provide full, adequate and complete history, I have obtained additional relevant history from caregiver who brought pt to office today. The history appears to be reliable. Immunization record reviewed.

HPI:

Generalized abdominal pain

Generalized abdominal pain noted. It began 6 days ago. It is localized primarily in the epigastric area and periumbilical area. It does not seem to radiate. She characterizes the pain as cramping and sharp. It is typically of moderate intensity. The discomfort occurs, on average, "comes in waves".. The duration is extremely variable. Associated symptoms include sour taste in mouth. She has not experienced abdominal distention or bloating, anorexia, constipation, dysuria, fever, myalgia, shortness of breath or vomiting. Nothing helps alleviate the pain. There are no known aggravating factors. Pt has constipation x several days and dad had been giving her N Acetyl Cysteine 600mg po q day and stopped 3 days ago, b/c read that NAC can cause constipation. Pt notes BM q day x 3 days and Bristol Type 4 or 5. No hard or large diam stools. No pain on defecation. No bld in or on stools. tsThere are no obvious stressors in her life. There is no evidence of school phobia. No family members or other contacts are similarly ill. [REDACTED] denies recent travel.

Nausea

Trichotillomania c/o pt continues to bite nails frequently as well as pulling out eyebrows frequently. Father states that through counseling "we have ruled out anxiety as a cause". Yet she has a diagnosis of Generalized Anxiety Disorder from psychiatrist. Pt on Sertraline for > 1year at 100mg but no change in dose with incr in sx of trichotillomania. Pt denies eating hair from body or head except eyebrows. Pt does eat fingernails and toenails and peeled dead skin from body. Denies eating paper, toilet or tissue paper, polyfil, stuffing, etc.

Major depressive disorder, single episode, moderate Dad notes that psychiatrist has retired and wants to have PCP manage meds. He does have appt at DFW Child Psychiatry in 1 month if PCP chooses not to manage meds. Pt has counselor appts x 1 mo. with someone who "also has trichotillomania" and "specializes in it". ts

Generalized anxiety disorder

Excoriation (skin-picking) disorder

Avoidant/restrictive food intake disorder Has not started Feeding Therapy or had evaluation by Childrens Health Eating Disorders Clinic. Dad states mom had info but did not make appt. He requests referral info to make appt himself. ts

Body mass index (BMI) pediatric, 5th percentile to less than 85th percentile for age

ROS:

CONSTITUTIONAL: Negative

E/N/T: Negative for ear pain, nasal congestion, frequent rhinorrhea and persistent sore throat.

RESPIRATORY: Negative for persistent cough and dyspnea.

INTEGUMENTARY: Negative for pruritis and rash.

Past Medical History / Family History / Social History:

Last Reviewed on 9/28/2024 07:54 PM by SCHLICHTEMEIER, TAMMI LYN

Past Medical History:

Positive for
 Major Depressive Disorder dx 2023 and
 Generalized Anxiety Disorder w Trichatillomania dx 2023; Hospitalizations: Never

CURRENT MEDICAL PROVIDERS:

PCP DR Schlichtemeier

Psychiatrist: Dr Hill, City View Psychiatry, Fort Worth

Surgical History:

NONE

Family History:

[REDACTED]

Social History:

Father: Christopher Copeland Parents' Marital Status: Divorced. lives with her father and has scheduled visits with her mother

Household: Lives with her father, sibling(s) (1 sister), and father's girlfriend and her 2 children..

No exposure to tobacco smoke.

Currently in Middle School. (CMS E; 7th grade; .She is performing fine at grade level and C in English or Math)

LYMPHATIC: no enlargement of cervical nodes;
SKIN: no ulcerations, lesions or rashes of exposed skin.

Assessment:

R10.13 Epigastric pain
DDx:
F63.3 Trichotillomania
DDx:
F32.5 Major depressive disorder, single episode, in full remission
DDx:
F41.1 Generalized anxiety disorder
DDx:
F42.4 Excoriation (skin-picking) disorder
DDx:
F50.82 Avoidant/restrictive food intake disorder
DDx:
Z68.52 Body mass index (BMI) pediatric, 5th percentile to less than 85th percentile for age
DDx:

ORDERS:

Meds Prescribed:

[New Rx] omeprazole 20 mg oral capsule, delayed release (enteric coated) [1 CAP PO Q DAY 30 MIN BEFORE DINNER ON EMPTY STOMACH.], #30 (thirty) capsules, Refills: 1 (one)

Procedures Ordered:

Other Orders:

3008F Body Mass Index (BMI), documented (PV) (In-House)
G2211 Complex e/m visit add on (x1)

Plan:

Epigastric pain. 1. Considering psych diagnoses and location of pain/ discomfort persistent, I cannot rule out gastric or duodenal ulcer, but do suspect GERD. 2. Rec trial of Omeprazole 20mg po q day x 2 wks. 3. If no improvement, plans to refer to GI Dr John Baker for possible endoscopy and H pylori breath test. 4. Call MD if increasing pain or increasing frequency. 5. RTC 2 wk recheck and prn. 6. Diet: No caffeine, carbonated drinks, chocolate, acidic foods, tomato, tomato sauce, cucumbers, pickles, garlic, MSG or spicy foods.

Prescriptions:

[New Rx] omeprazole 20 mg oral capsule, delayed release (enteric coated) [1 CAP PO Q DAY 30 MIN BEFORE DINNER ON EMPTY STOMACH.], #30 (thirty) capsules, Refills: 1 (one)

Orders:

Trichotillomania. 1. At some point, pt had GAD, which may well have played a role in her trichotillomania onset. 2. Now she may have sx due to anxiety or due to habit or environmental stressors. 3. Suspect GAD may play role in possible gastritis currently. 4. Continue Sertraline 100mg po qhs.

Major depressive disorder, single episode, in full remission. 1. No sx of depression in over 1 year per pt and dad. 2. Will consider change to Escitalopram for coverage of anxiety and see if trichotillomania improves and if no, then may want to see if any depressive or GAD sx recur. ts

Generalized anxiety disorder

Excoriation (skin-picking) disorder

Avoidant/restrictive food intake disorder. 1. Gave father referral info for Children's Health Eating Disorders Clinic. 2. Difficult to increase wt gain due to ARFID. ts

Body mass index (BMI) pediatric, 5th percentile to less than 85th percentile for age

Orders:

3008F Body Mass Index (BMI), documented (PV) (In-House)

Charge Capture:

Primary Diagnosis:

R10.13 Epigastric pain

Orders:

99214 Established patient outpatient visit, Moderate MDM and/or 30-39 minutes (33 minutes) (In-House)
G2211 Complex e/m visit add on (x1)

F63.3 Trichotillomania
F32.5 Major depressive disorder, single episode, in full remission
F41.1 Generalized anxiety disorder
F42.4 Excoriation (skin-picking) disorder
F50.82 Avoidant/restrictive food intake disorder
Z68.52 Body mass index (BMI) pediatric, 5th percentile to less than 85th percentile for age

Orders:

3008F Body Mass Index (BMI), documented (PV) (In-House)

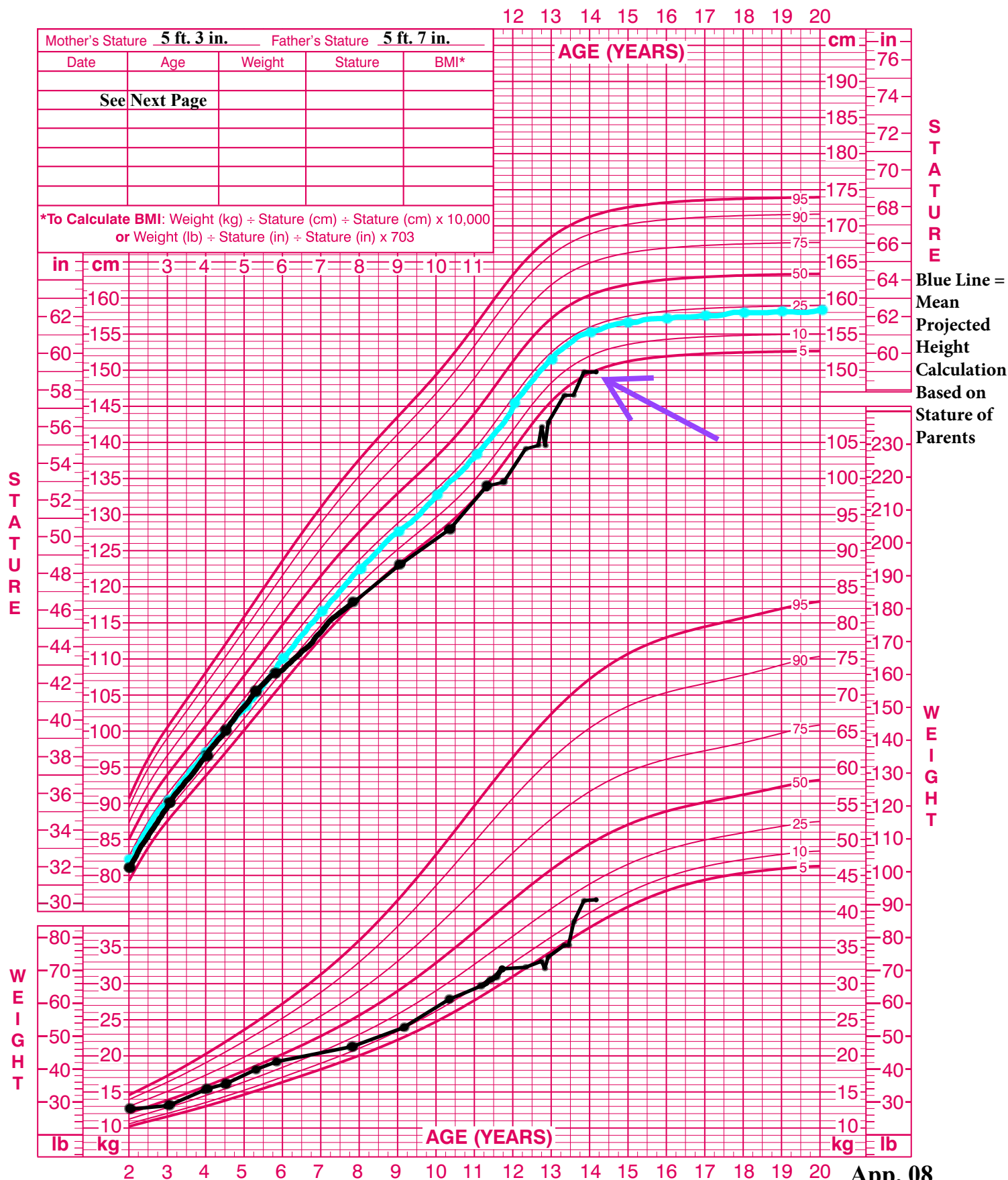
2 to 20 years: Girls Stature-for-age and Weight-for-age percentiles

NAME

[REDACTED]

Copeland

DOB: X/X/2012



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Published May 30, 2000 (modified 11/21/00).

SOURCE: Developed by the National Center for Health Statistics in collaboration with the National Center for Chronic Disease Prevention and Health Promotion (2000).

<http://www.cdc.gov/growthcharts>



SAFER • HEALTHIER • PEOPLE™

Date	Age	Weight	Percentile	Height	Percentile	BMI	Percentile
2/28/2014	2 years	12.87 kg	72 %	81.28 cm	14 %	19.48	97 %
3/17/2015	3 years	13.15 kg	30 %	90.17 cm	14 %	16.18	65 %
3/30/2016	4 years 1 month	15.88 kg	48 %	96.52 cm	13 %	17.04	88 %
8/6/2016	4 years 5 months	16.6 kg	48 %	100.08 cm	20 %	16.58	82 %
6/26/2017	5 years 4 months	18.28 kg	44 %	105.41 cm	17 %	16.45	80 %
12/5/2017	5 years 9 months	19.19 kg	43 %	107.95 cm	15 %	16.47	78 %
12/19/2019	7 years 9 months	21.41 kg	15 %	118.11 cm	6.6 %	15.35	41 %
3/25/2021	9 years 1 month	23.81 kg	11 %	123.19 cm	4.7 %	15.69	37 %
1/11/2022	9 years 10 months	22.68 kg	1.6 %	124.46 cm	2.2 %	14.64	12 %
6/14/2022	10 years 3 months	27.71 kg	12 %	128.27 cm	4.5 %	16.84	47 %
6/14/2023	11 years 3 months	30.62 kg	11 %	134.62 cm	5.9 %	16.89	38 %
10/5/2023	11 years 7 months	31.48 kg	9.9 %	135.38 cm	4.1 %	17.18	40 %
12/5/2023	11 years 9 months	32.21 kg	10 %	134.62 cm	2.3 %	17.77	48 %
7/3/2024	12 years 4 months	32.39 kg	5.3 %	139.06 cm	2.4 %	16.75	26 %
10/24/2024	12 years 8 months	31.81 kg	2.6 %	139.7 cm	1.4 %	16.3	17 %
11/27/2024	12 years 9 months	33.11 kg	4.0 %	142.24 cm	2.7 %	16.36	17 %
1/2/2025	12 years 10 months	32.18 kg	2.2 %	139.7 cm	0.9 %	16.49	18 %
1/31/2025	12 years 11 months	33.84 kg	4.2 %	142.88 cm	2.4 %	16.58	19 %
7/3/2025	13 years 4 months	35.29 kg	4.3 %	146.69 cm	4.0 %	16.4	14 %
8/6/2025	13 years 5 months	35.29 kg	3.8 %	146.69 cm	3.5 %	16.4	13 %
10/3/2025	13 years 7 months	38.65 kg	10 %	146.69 cm	2.9 %	17.96	33 %
1/5/2026	13 years 10 months	41.46 kg	17 %	149.86 cm	6.1 %	18.46	39 %
4/6/2026	14 years 2 months	41.912 kg	16%	149.86 cm	5%	18.4	40%

Copeland

Sex: Female
Birthdate: 12
Last Physical: 07/03/25
Race: White
Ethnicity: Not Hispanic or Latino
Preferred Language: English

Visit Summary for 04/06/26

Tammi Schlichtemeier, MD
EmPower Children's Clinic, PA
Sick

Diagnoses

Generalized anxiety disorder
Pure hypercholesterolemia
Trichotillomania

Vitals

Weight
92 lbs 6.4 oz (41.912 kg)
16th percentile (CDC)

Height
59 in (149.86 cm)
5th percentile (CDC)

Blood pressure
106 systolic 71 diastolic

Temperature
97.8° F

Pulse
81 beats per min

Respiratory rate
14 breaths per min

O2 saturation
97 percent

BMI
18.7 kg/m²
40.0th percentile (CDC)

Other Health Information as of 05/04/26

Medications

Active

atomoxetine (18 mg Capsule)
Instructions: 1 Capsule By Mouth every morning
Start: 04/01/26

sertraline (100 mg Tablet)
Instructions: 1 Tablet By Mouth every day at bedtime
Start: 04/08/26

sertraline (25 mg Tablet)
Instructions: 1 Tablet By Mouth every day at bedtime
Start: 04/08/26

Allergies

Active

No known allergy
Onset: Unspecified

No Known Drug Allergies

Problem List

Active

Family disruption with separation
Onset: 07/07/24

Major depression in remission
Onset: 07/07/24

Generalized anxiety disorder
Onset: 07/07/24

Acute excoriation of skin

Compassionate Treatment for Children with Avoidant/Restrictive Food Intake Disorder (ARFID)

Children's Health's ARFID treatment program is one of the most comprehensive and successful in the U.S. Our **4-week program** will help your child overcome their struggles with food so they can lead a life free from nutrition-related problems.

ARFID in children is an eating disorder in which a child imposes intense limits on the types of food they will eat and how much food they will eat. Though Avoidant/Restrictive Food Intake Disorder seems like anorexia, children with ARFID aren't worried about their body image, size or weight. Rather, they are so picky that their eating habits affect their health, growth, social life and family life.

ARFID often starts when a child is young. It can affect their growth and development because they don't get all the nutrients they need. It can also affect a child's health, mood, behavior and energy level.

Our experts at Children's HealthSM have helped many children overcome ARFID. We use proven treatments to help them embrace new foods and get them on track for a healthy future.

Contact us

New Patient: ☎ 214-456-8899
Fax: 469-303-4095
Ste P4300

Plano

Phone: ☎ 469-303-4787
Fax: 214-456-5941
Ste P4300

World-renowned ARFID experts

Our center partners with UT Southwestern, one of the nation's premier teaching hospitals and healthcare research centers. As part of our ARFID program, your child will be treated by top psychiatrists and psychologists who specialize in ARFID and other eating disorders. **Our team is so renowned, other doctors and mental health professionals come to learn from us at our annual eating disorder symposium.**

4-Week Outpatient Program for Children Ages 5 to 17

We offer a 4-week, outpatient treatment program for children ages 5 to 17. Children attend the program Monday to Friday for about the length of a school day. Over the course of 4 weeks, your child will benefit from:

- **Individual and family therapy** to address behavior and eating habits, stressful or traumatic events, and identify any other mental or physical issues. We use games and fun activities to connect with younger children. And we help your family create a behavior plan to inspire your child to expand the foods they eat.
- **Group therapy** to help children and teens support one another as they overcome fears around food and eating. We have different groups for younger children and teens.
- **Support for school and homework** with our certified teachers who work with your child for 2 hours a day to make sure they stay current with their schoolwork during treatment.
- **Occupational therapy** to identify the textures, odors, temperatures and colors that may trigger your child, and gradually work with them to get better at identifying and trying new foods.
- **Experiential therapy** to help your child connect to their body and feelings through art, music and recreation therapy.

App. 11

🗣 Need Help?

- **Meal therapy** to help your child slowly experience new foods, one bite at a time. We have the patience and expertise to overcome spitting out, crying or delays at mealtimes. We can empathize with the anxiety that eating certain foods can cause.
- **Medication management** to help with problems like anxiety, obsessive compulsive disorder, depression, self-injury, or post-traumatic stress disorder. These problems can contribute to ARFID.

As a parent, you'll also receive a lot of helpful training and support from the Children's Health team. For a few hours each week you will attend:

- **Parent skills group** to teach you how to set limits and motivate your child. We'll empower you to continue the work of expanding what your child eats. We'll help you feel confident that your child is getting what they need. And we'll practice ways to stay calm and relaxed when you need to.
- **Multi-family therapy** to help you connect with other families experiencing ARFID, so you can share tips, tricks and struggles.

World-renowned doctors and psychologists

Our center collaborates closely with UT Southwestern, one of the nation's premier teaching hospitals and healthcare research centers. As part of our ARFID program, **your child will be treated by some of the top psychiatrists and psychologists who specialize in eating disorders.** Doctors and mental health professionals from around the U.S. come to our annual eating disorder symposium to learn from us each year.

A state-of-the-art facility

We know that a pleasant environment can help people feel better and more relaxed. That's why we've designed our facility to be bright, fun and inviting. Our facility includes outdoor spaces, recreational spaces, and therapy rooms that look out onto our gardens. Your child can read, play games and listen to music in rooms that are designed with them in mind.

Our ARFID program is separate from our other eating disorder programs

We are the only program in the country that has an ARFID program that's completely separate from our other eating disorder programs. We keep the ARFID program very small so that children and families get the care and attention they need so they can get on a better path in only 4 weeks.

We provide treatment and support for the whole family because we know that eating is an important, lifelong family ritual. Families develop bonds with other ARFID families that often last beyond the program. And they leave with a new enthusiasm for their own family.

Meet the Care Team



Urszula Kelley, MD
Pediatric Psychiatrist



M Elizabeth Weidmer-Mikhail, MD
Pediatric Psychiatrist



Mohanika Gowda, MD
Pediatric Psychiatrist

Copeland

Sex: Female
Birthdate: 15
Last Physical: None
Race: White
Ethnicity: Not Hispanic or Latino
Preferred Language: English

Visit Summary for 05/04/26

Tammi Schlichtemeier, MD
EmPower Children's Clinic, PA
Sick

Vitals

Weight
58 lbs 2 oz (26.365 kg)
3rd percentile (CDC)

Height
53.5 in (135.89 cm)
15th percentile (CDC)

Blood pressure
91 systolic 60 diastolic

Temperature
97.5° F

Pulse
92 beats per min

Respiratory rate
18 breaths per min

BMI
14.3 kg/m²
4.0th percentile (CDC)

Labs

Rapid Strep (in office)
Streptococcus pyogenes Ag [Presence] in Throat by Rapid immunoassay
Interpretation: **Normal**
Result: Negative

Other Health Information as of 05/04/26

Medications

None

Allergies

Active
No known allergy
Onset: Unspecified

Problem List

Active
Adjustment disorder with anxious mood
Onset: 02/02/25
Allergic rhinitis caused by pollen
Onset: 08/25/25

REVIEW

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Unique considerations for the medical care of restrictive eating disorders in children and young adolescents

Anna B. Tanner^{1,2*}

Abstract

Background The medical complications of eating disorders are often approached through an age-neutral lens. However, children and young adolescents may have unique medical complications related to the energy needs and timing of growth and development. Providers caring for patients in this vulnerable age range should understand how to identify, approach, and manage these potential age-related complications.

Review Evidence continues to accumulate that increasingly younger patients are being diagnosed with eating disorders. These children and young adolescents have significant risk for unique and potentially irreversible medical complications. Without early identification and treatment, restrictive eating disorders may negatively impact linear growth, bone development and brain maturation in children and young adolescents. Additionally, due to the energy needs of growth and development, unique considerations exist for the use of acute medical stabilization and the identification of patients at risk for refeeding syndrome with initial nutritional rehabilitation. This review presents an approach to the evaluation and management of children and young adolescents with eating disorders.

Conclusion Children and young adolescents with restrictive eating disorders may have unique medical complications related to the energy needs and timing of linear growth and pubertal development. Significant risk exists for irreversible medical complications of impaired growth, bone, and brain health. Increased awareness of the energy needs for growth and development may improve early recognition, appropriate intervention, and future outcomes for children and young adolescents with restrictive eating disorders.

Keywords Eating disorders, Children and young adolescents, Medical complications, Growth and development

Plain English summary

Eating disorders are affecting increasingly younger patients, with ages of onset that now include children and young adolescents. This brings new risk, as children and young adolescents with eating disorders may develop unique medical complications related to the energy needs for the growth that should occur during this developmental stage. Inadequate energy intake during eating disorders in children and young adolescents can have long-term consequences including negative impacts on final height and bone and brain health. Children and young adolescents with eating disorders may present to care with acute concerns that require medical hospitalization and significant support to begin feeding again safely. Providers caring for children and young adolescents with eating disorders should be

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familiar with these unique complications and understand critical factors in the initial assessment, medical stabilization, and nutritional rehabilitation of children. The clock is ticking, and the goal of care of children and young adolescents with restrictive eating disorders should be to safely restore normal growth and puberty as quickly as possible.

Background

Providers caring for the youngest patients in eating disorders care, specifically children and young adolescents, must recognize that critical differences exist in the medical evaluation and management of this population from that of older adolescents and adults. Current epidemiological trends and expanded recognition of eating disorders through new and revised diagnostic criteria have brought increasing numbers of younger patients to eating disorders care. Medical complications, due to restrictive behaviors, may present differently in children and young adolescents, who are pre-pubertal or peri-pubertal and have additional biological vulnerability due to the critical energy needs of growth and development.

Unique medical complications, related to linear growth, bone development and brain maturation, may be irreversible unless detected and addressed with effective early intervention. Heightened recognition of these developmentally based medical concerns and an increased understanding of current recommendations for acute medical management will improve outcomes and decrease morbidity and mortality associated with these illnesses in children and young adolescents with eating disorders.

Current epidemiological trends of eating disorders in children and young adolescents

Eating disorders are common in children and adolescents, with increasingly younger patients affected. Estimates of the prevalence of all eating disorders in adolescents are 6–8% [1], although rates may be increasing as indicated by rising numbers of medical admissions for acute stabilization noted during the COVID-19 pandemic [2]. Recent studies show an increase in incidence of Anorexia Nervosa (AN) in younger patients, and a mean age of diagnosis of 12.2 years of age [3, 4]. This is consistent with prior studies that described an average age of onset of 12.3 years of age for AN [5]. In younger patients, males may represent a higher percentage of eating disorders patients than previously recognized, with one recent study showing that 24% of young patients presenting with AN were male [4].

Children and young adolescents can be affected by any eating disorder diagnosis. Revisions of diagnostic criteria in the Diagnostic and Statistical Manual of Mental Disorders (DSM-5) [6], have helped to clarify eating disorder

diagnoses for many younger patients. New diagnoses, such as Atypical Anorexia Nervosa (AAN) and Avoidant/Restrictive Food Intake Disorder (ARFID) are seen frequently in children and young adolescents. Patients meeting criteria for ARFID comprised 14% of patients presenting for eating disorder care in one multicenter study [7]. In another study, one third of acute inpatient medical admissions for patients with restriction were patients with AAN [8]. Despite changes, substantial numbers of adolescents with disordered eating are still not receiving treatment [9].

Energy needs for growth and development create a period of nutritional vulnerability [10], which may produce a disparate medical impact in younger patients with restrictive eating disorders. Although all eating disorder diagnoses can be seen in children and young adolescents, the medical complications of restrictive eating disorders may generate urgent and potentially irreversible medical complications. Challenges exist in identifying and addressing unique medical complications for young patients with any restrictive behaviors, but predominantly those with AN, AAN and ARFID diagnoses. For children and young adolescents, unique medical complications related to restriction prevail and will be the primary focus of the remainder of this review.

The initial medical evaluation in children and young adolescents

Treating increasingly younger patients with restrictive eating disorders brings new challenges in illness detection and medical management, as well as new risk due to the considerable energy needs and potential for missed opportunities for normal growth and development. Childhood and adolescence are a time-limited window for linear growth, bone development and brain maturation. When faced with starvation, growth failure and stunting may result [11]. Children should be growing, and adolescents should be in puberty. When restrictive eating disorders disrupt these critical developmental tasks, irreversible medical complications may result.

In adults and older adolescents, growth and puberty are finished, and patients with restriction may present overtly with weight loss accompanied by laboratory or vital sign abnormalities, amenorrhea or symptoms of low testosterone, and demonstrate medical compromise from starvation across organ systems. However, in children and young adolescents, growth and puberty are

not complete. With eating disorders in prepubertal and peripubertal children and adolescents, growth retardation and pubertal delay may occur [12]. Medical evaluations must include discerning assessments of growth and puberty, and not rely solely on detecting overt weight loss, abnormal laboratory values, abnormal vital signs, amenorrhea or other symptoms of low hormone levels, to facilitate early detection and early intervention in younger eating disorders patients.

As in the initial medical evaluations in older adolescents and adults, a thoughtful and restrained rule out of other primary medical and psychiatric issues is prudent. However, providers must remember that extensive evaluations to rule out other causes may inadvertently delay initiation of care, resulting in increased severity of medical concerns. Anecdotally, these delays may also increase reluctance of patients and families to accept an eating disorders diagnosis when it is finally offered. Diagnosing younger patients with eating disorders can be difficult, as the link between eating disorders behaviors and presenting medical concerns may be less obvious. Restricted intake can be surreptitious and may result in lack of expected weight gain rather than obvious weight loss [13]. In younger patients, medical concerns such as constipation, headaches or dizziness may present before weight or intake changes are noted. Thus, medical concerns may inadvertently be perceived as the cause of the weight loss and restricted intake.

Per recommendations from the American Academy of Pediatrics (AAP), the initial medical evaluation of all children and young adolescents with known or suspected eating disorders should include an evaluation of growth parameters [12]. In addition to vital signs and laboratory studies, in young patients it is also useful to collect height and weight, determine pubertal status, review growth charts, and calculate mid-parental height (MPH). Current height can be compared to genetic potential using MPH, which may be calculated with the following formula:

- For biologic females (Mother's height (in) + Father's height (in) – 5)/2
- Or (Mother's height (cm) + Father's height (cm) – 13)/2
- For biologic males (Mother's height (in) + Father's height (in) + 5)/2
- Or (Mother's height (cm) + Father's height (cm) + 13)/2
- Predicted range ± 3 in or 8.5 cm [14].

Growth charts should be reviewed and assessed in the context of growth patterns prior to illness onset. In malnourished children and young adolescents, a drop

in weight percentile will be followed by a drop in height percentile on growth curves if energy deficiency is not amended. Shifts across two or more percentiles are uncommon in healthy growth [14]. The primary treatment goal for all young patients with restrictive eating disorders is to restore growth to prior linear growth channels and to maximize growth to genetic potential. Variances noted in growth curves may be used as a motivational interviewing tool in initial discussions with patients and parents.

Assessing severity of weight loss and malnutrition is crucial to adequately determine the impact of restriction and to appropriately direct initial steps for intervention, including identifying a need for weight restoration. For children and adolescents, weight restoration to support normal growth and development is an important goal of early treatment [15]. Although body mass index (BMI) is a frequently used tool in adult assessments, it is a less useful tool in younger patients when determining degree of malnutrition at presentation. The Society for Adolescent Health and Medicine (SAHM), proposes that malnutrition in children and adolescents be classified as mild, moderate, or severe, and factors to consider should include percent and rate of weight loss, percent median BMI (%mBMI), and BMI Z-score. For children and adolescents, SAHM proposes classifying severe malnutrition as %mBMI < 70%, BMI Z-score ≤ -3 , loss of more than 10% of body mass, or rapid weight loss (defined as > 5% in month, > 7.5% in 3 months, > 10% in 6 months or > 20% in one year) [15].

In children and young adolescents, changes in BMI Z-scores may be a useful indicator of malnutrition. Changes in BMI Z-scores may be used to evaluate both total weight loss and recency of weight loss, which have been shown to predict risk of medical complications in children and young adolescents [8]. In addition, providers must remember that children and young adolescents may have decreased BMI Z-scores without any overt weight loss. BMI Z-scores will decrease when growing children fail to gain adequate weight while still increasing linear height. Constant mindfulness of normal growth and pubertal development, as well as use of age-appropriate markers of malnutrition are critical in the appropriate initial assessment of children and young adolescents with restrictive eating disorders [15].

Impact of restrictive eating disorders on linear growth

The potential for eating disorders to disrupt linear growth in children and adolescents has been well described [13, 16]. Detection of deceleration of linear growth has been identified as a critical factor in the early identification of eating disorders in younger patients [13]. Supporting

normal growth is a critical goal of nutritional rehabilitation [15]. Progression to skeletal maturity may limit potential adult height if weight restoration is delayed. Final adult heights shorter than that predicted by mid-parental height have been documented in adolescents with anorexia nervosa [17, 18]. Attention to growth in restrictive eating disorders is important, and weight restoration must occur to limit the final impact on adult linear height.

Children should be growing, and for young patients, growth curve deviations may alert providers that a restrictive eating disorder is present before parental suspicion is aroused. One study demonstrated growth curve deviations in 48% of adolescent eating disorders patients an average of 9.7 months before parents reported symptoms. Patients with slower weight loss were more likely to have symptoms noted after growth chart deviation had occurred, compared to patients with more acute weight loss [13]. Astute attention to growth curves may provide earlier detection of eating disorders in children and young adolescents and limit impact on final height.

Recent reviews demonstrate that catch-up growth may be suboptimal and may not be seen in all young patients with restrictive eating disorders. In a systematic review and meta-analysis of 27 studies, patients presenting at a younger age or with longer duration of illness were at greater risk for growth delay [16]. The authors emphasize that attention to growth “is important, especially in the young.”

Energy insufficiency will have a greater effect on growth when it is prolonged or occurs during times of rapid growth potential. Take off growth for biologic females occurs at age 9, with peak height velocity occurring at age 11, and for biologic males, these events occur 2 years later. The growth spurt accounts for 17–18% of final adult height [14]. When energy insufficiency occurs during these time limited developmental windows, final linear growth is impacted, as the opportunity to gain linear height is lost.

Goals for catch-up growth must be evaluated against genetic potential, by assessment of parental height, as well as growth curves [15]. Both male and female adolescents with AN are at risk for final adult heights lower than predicted by mid-parental target height [17, 18]. Poor catch-up growth may be an increasing concern as more patients present at younger ages. The AAP highlights that younger patients are at risk for greater and more permanent effects on growth [12]. Growth potential is lost as patients age. Weight restoration to support individual pre-morbid trajectories of linear growth to prior growth channels and to genetic potential is an important primary medical goal in all young patients presenting with restrictive eating disorders [15].

Impact of restrictive eating disorders on bone development

Restrictive eating disorders have been identified as a risk factor for decreased bone mineral density (BMD) in adolescents. Peak bone mass is acquired during healthy adolescence, thus placing young patients with restrictive eating disorders at unique risk for poor bone health. Reductions in BMD in adolescents with AN have been well demonstrated [19]. Additionally, no significant sex differences in bone deficits are seen between male and female adolescents with AN, when controlling for age, duration of illness and %mBMI [20]. Impaired bone health has also been demonstrated in underweight adolescents with ARFID [21]. In adolescents with AAN, disordered bone health has also been described [22]; however additional studies suggest that a prior history of overweight may be associated with higher BMD Z-scores than patients with AN [23]. For adolescent athletes, weight bearing exercise and team sports may have some protective effect on BMD, however any benefits of exercise must be balanced against other risks in the management of the eating disorder [24].

In contrast to adult patients with AN, where decreases in BMD are due to both decreased formation and increased resorption, low bone density in young patients with anorexia nervosa primarily represents a missed opportunity for bone deposition. Significant bone mass (40–60%) is acquired in adolescence and multiple factors including low body weight, estrogen and testosterone deficits, GH resistance, low IGF-1 levels, and elevated cortisol levels may contribute to low bone mass [25]. In a longitudinal study, persistent negative effects on bone health were demonstrated in females with AN as adolescents despite recovery in body weight and BMI [26]. The development of AN during the adolescent years interrupts a critical window for bone mass acquisition. Understanding the potentially irreversible impact of restrictive eating disorders on bone health may serve as a “wake up call” for patients and parents [27]. Providers caring for children and young adolescents must convey that risk exists for poor bone density outcomes when restrictive eating disorders remain unchecked.

Sex hormones have a significant impact on bone health and are often impacted in restrictive eating disorders [25]. Adolescents should be in puberty, and appropriate assessment of pubertal onset and current sexual maturity in adolescents with restrictive eating disorders is a critical task [15]. Functional hypogonadism may occur in patients with poor nutrition and anorexia nervosa [28]. Biologic female adolescents typically have the onset of puberty a year before biologic males [28], with menarche between 12 and 12.5 years of age [29]. An assessment for other pubertal milestones, such as thelarche and

pubarche [29], as well as a menarchal history should be collected in all biologic females at presentation. Biologic males typically begin puberty between ages 9 and 14, and will have milestones that include testicular enlargement, penile length increase, and development of pubic hair. Facial hair and voice deepening are later changes [29]. Providers may document physical exam findings of puberty by using a Sexual Maturity Rating (SMR) [29].

Providers should complete a thorough pubertal history on all eating disorder patients and complete an assessment of current sexual maturity [15]. In older adolescents and adults, commonly recognized symptoms of amenorrhea or decreased libido may indicate decreased estrogen or testosterone levels. In children and younger adolescents, malnutrition may present only as delayed or interrupted puberty, or later age of menarche [29], which may generate less explicit concern. This may inadvertently place younger adolescents at higher risk for poor bone health, especially adolescents in earlier stages of puberty, those with more significant pubertal delay, and those with longer pubertal suppression. In one study highlighting the risk of early onset AN, (AN onset prior to age 14 years old), patients with the most pubertal delay were found to have greatest negative impact on bone mineral content [30].

Updated guidelines for assessing bone health in adolescents with restrictive eating disorders are lacking. A recent international panel on child and adolescent bone health continued to define the need for dual energy X-ray absorptiometry (DXA) for patients with AN in terms of amenorrhea, recommending it for all patients with AN and amenorrhea for more than 6 months [31]. The AAP highlights that reduced BMD is related to malnutrition, as well as duration of amenorrhea in girls and low testosterone in boys. However, the AAP recommendations on when providers should evaluate bone health by DXA defers to children with clinically significant fractures and patients with medical conditions that place them at increased risk of fractures [25]. Among providers caring for adolescents with eating disorders, the lack of updated consensus guidelines has led to use of clinical factors in the decision-making process for young patients with restrictive eating disorders. Although one study found no difference in the rates of obtaining DXA scans for male and female patients, different clinical factors were identified in the decision-making process. Medical providers in this study used history of fractures, degree of malnutrition and duration of illness when deciding to order DXA scans for male patients and amenorrhea, history of fractures and duration of illness for female patients [32].

Recommendations from SAHM review that low bone mineral density occurs in adolescent patients with AN and that the optimal management remains unresolved

[33]. Oral combined estrogen-progestin has not been found to increase BMD and is not recommended in adolescent females. Bisphosphonates are not currently recommended for adolescents with eating disorders and decreased BMD. One study on the use of transdermal estrogen did demonstrate increased spine and hip BMD scores [34]. Current SAHM guidelines emphasize weight restoration with spontaneous resumption of menses, optimization of calcium and vitamin D intake, and treatment of vitamin D deficiency if present [33]. For males, these recommendations could be extended to emphasize weight restoration with normalization of age-appropriate testosterone levels, optimization of calcium and vitamin D intake, and treatment of vitamin D deficiency if present.

Impact of restrictive eating disorders on brain maturation

Although perhaps less apparent and more difficult to assess, restrictive eating disorders may have a deleterious impact on adolescent brain development. Part of normal childhood growth and adolescent development is critical brain growth and development. The child and young adolescent brain may be more vulnerable to the impact of starvation seen in patients with restrictive illnesses such as anorexia nervosa. Functional and structural changes, including changes to grey and white matter, have been observed in the brains of adolescents with eating disorders [35].

In adolescents with AN, loss of grey matter has been shown to be greater than that in adults. While these changes have been demonstrated to improve in adults with weight restoration, long-term studies demonstrating improvement in adolescents have been lacking [36]. Grey matter reduction in adolescents may be due to nutritional deficiencies resulting in neuronal apoptosis. Additionally, malnutrition may modify the process of dendritic pruning, a normal part of brain development in the maturing adolescent brain. These changes be an attempt to save energy during starvation and may explain the greater grey matter losses observed in adolescents. In patients with a diagnosis of AAN, no volumetric differences were found leading some researchers to hypothesize that a critical BMI threshold exists below which gray matter loss becomes apparent [35].

Additional studies evaluating white matter in adolescents with AN have found differences in microstructure by diffusion-weighted imaging, consistent with changes in myelination. These changes have been shown to potentially reverse with short-term weight restoration [37]. Similar changes to white matter have not been demonstrated in adolescents with AAN, further supporting the

association between severe weight loss and alterations in white matter [38].

The dendritic pruning and increased myelin production that occur during healthy adolescence are associated with development of abstract thinking and executive functions. Cognition and reward pathways evolve during adolescence under the influence of reproductive and stress hormones. With starvation, brain changes occur over time and individuals may remain stuck in the adolescent brain and become more resistant to change. Early intervention for anorexia nervosa in children and young adolescents may mitigate the impact of starvation on the developing brain and minimize the duration of impact on neurodevelopment [39].

Medical complications in children and young adolescents with restrictive eating disorders

Every organ system may be impacted by starvation in children and young adolescent patients with restrictive eating disorders. Children and adolescents are at risk for significant neurologic, cardiovascular, gastrointestinal, hematologic, and endocrine complications related to dietary restriction or weight loss [12], in addition to the unique concerns related to linear growth, bone development and brain maturation discussed previously.

Rate and percentage weight loss are important risk factors for many of the observed medical complications in children and adolescents. Renal impairment was shown in 33% of adolescents hospitalized for medical instability due to AN and AAN in one study. Patients presenting with more rapid weight loss were more likely to have renal impairment, suggested by a decreased estimated glomerular filtration rate (eGFR) [40]. In another study, elevated lipase levels, without evidence of pancreatitis, were identified in 34% of adolescent patients with AN or AAN presenting for inpatient treatment. Higher percentage of weight loss and lower %mBMI at presentation were associated with higher lipase levels [41].

Weight suppression, defined as the difference between premonitory and presentation weight, has been studied as a factor in medical complications in adolescent patients with AAN [42]. Total weight loss, as well as recency of weight loss, have been shown to be better predictors of many medical complications than presentation weight [8]. Similar rates of hypophosphatemia [43], bradycardia and orthostatic instability have been described in hospitalized adolescents with AN and AAN [44]. One study of adolescents with AN and AAN showed specifically that lower heart rate was associated with faster weight loss, and lower serum phosphorous levels were associated with greater amounts and longer durations of weight loss [42]. Despite these significant medical concerns, recent studies demonstrate decreased referrals of patients with

AAN to eating disorder specific care [45], and lower odds of adolescent patients with AAN receiving inpatient medical care [46]. Guidelines for assessing medical severity should continue to incorporate weight suppression to improve early detection and minimize medical complications.

Biologic male adolescents with restrictive eating disorders are not spared significant medical complications and may also face barriers in accessing adequate medical care. In one study, 39% of adolescent males had bradycardia that met SAHM hospital admission criteria at the time of presentation [47]. Progression to severe cardiovascular compromise may occur prior to eating disorder diagnosis. Recent studies have demonstrated that hospitalized adolescent males may also have increased medical needs once admitted for initial refeeding, including higher estimated energy requirements, higher caloric needs at discharge, longer admissions, and greater weight changes during medical stabilization. Associated factors included lower admission weight and lower admission heart rate, indicating increased medical severity by the time the eating disorder was identified [48]. Once an eating disorder is identified in an adolescent male, providers may be also less likely to screen for hypothalamic–pituitary–gonadal (HPG) axis suppression, with less frequent assessment of SMR, libido, or laboratory studies in males [49]. Guidelines for assessing medical severity and risk may need to be revised and adapted to adequately assess male adolescents with restrictive eating disorders, to improve earlier detection, and minimize medical complications.

Severe medical complications in children and adolescents with ARFID have been well documented and include significant eye disorders related to vitamin A deficiency, scurvy related to vitamin C deficiency, and anemia related to iron deficiency [50, 51]. Children and adolescent patients with ARFID have also been shown to have lower intake of vitamins K and B12 related to limited vegetable and protein intake [52], and deficiencies in thiamine and vitamin D [51]. Failure to make appropriate gains in weight and height during childhood and adolescence are known presentations [53], and weight loss and faltering growth are considered defining characteristics of ARFID [54]. However, significant nutritional deficiencies may be present even in ARFID patients that are not underweight and should not be missed [51]. Recent studies have shown similar medical comorbidities and endocrine conditions may exist in patients with AN and in patients with low weight ARFID, leading to calls from researchers to treat low weight ARFID as intensively as AN [55]. However, low intake may be chronic in patients with ARFID and associated with less bradycardia and less amenorrhea than in children and adolescents with AN and AAN, leading to less admissions for acute

medical stabilization [56]. Guidelines for assessing medical severity may need to be revised and adapted with these findings in mind, to adequately assess children and adolescents with ARFID, to improve earlier detection, and to minimize medical complications.

Special considerations must be taken for the management of adolescents with disordered eating and gender identity concerns. Studies have shown higher prevalence of some eating disorder behaviors in gender nonconforming adolescents, in particular fasting in gender nonconforming males [57]. Due to concerns for potential restriction, screening for malnutrition in transgender youth has been advocated. For children and young adolescents with growth potential, specific challenges exist for evaluating and monitoring growth in the context of both disordered eating and gender transitioning. Consensus recommendations for monitoring growth in transgender individuals are lacking and adapted growth curves currently do not exist [58]. SAHM states that determination of weight goals in transgender adolescents “is particularly challenging” [15]. A recent case series reporting disordered eating among transgender youth with autism spectrum disorder highlights that care for these patients may be complex and require an individually tailored approach [59]. Current recommendations for adult transwomen and transmen identify low body weight as a risk factor for decreased bone health [60], adding further complexity and importance to identify standards for setting weight goals and monitoring growth in this population. Patients may need multidisciplinary teams for both gender-affirming care and eating disorder specific care and an individually tailored approach, especially when growth is a concern [59]. Once again, current guidelines to assess medical severity may need to be revised and adapted to adequately assess children and adolescents with eating disorders and gender concerns, to improve earlier detection and minimize medical complications.

Acute medical stabilization in children and adolescents with eating disorders

At initial presentation, all patients with a known or suspected eating disorder must undergo an evaluation for medical stability and factors supporting acute medical hospitalization of children and adolescents are well established [15]. Children and young adolescents may be weight suppressed without overt weight loss and may become medically unstable rapidly due to the energy demands of growth and development. At presentation, patients may have depressed vital signs, significant electrolyte disturbances and electrocardiogram (ECG) abnormalities that necessitate admission for acute medical stabilization. Cardiovascular complications, including

bradycardia, hypotension, and orthostatic vital signs changes, are frequently used indicators of need for medical stabilization in restrictive eating disorders in children and young adolescents. Guidelines from SAHM have provided criteria for inpatient admission for acute medical stabilization in children and adolescent [15].

Updated criteria from SAHM reinforce past recommendations for acute medical stabilization for children and young adolescents for concerns requiring immediate medical intervention or monitoring, including electrolyte disturbances, electrocardiogram abnormalities, and dehydration [15]. Vital sign abnormalities that support hospitalization include severe bradycardia (HR < 50 BPM daytime), hypotension (BP < 90/45 mm Hg), hypothermia (body temperature < 96 °F, 35.6 °C), or orthostatic vital sign changes (defined as sustained increase in HR > 40 bpm in adolescents under age 19 years or sustained decrease in blood pressure of > 20 mm Hg systolic or > 10 mm Hg diastolic) [15].

Bradycardia is a commonly used marker of instability in restrictive eating disorders in children and young adolescents. Longer duration of bradycardia and longer lengths of stay have been associated with lower presenting heart rates in adolescents with AN [61]. Thus, the recommendation to admit pediatric and adolescent patients with a heart rate below 50 beats per minute for acute medical stabilization remains unchanged in the updated guidelines [15].

Orthostatic vital sign changes have also been frequently used as a determinate of need for acute medical stabilization in children and adolescents. A significant update in the most recent criteria from SAHM allows for increased orthostatic heart rate and blood pressure changes than previously recommended, permitting orthostatic changes of 40 beats per minute in patients under 19 years of age compared to the 20 beats per minute that was included in all prior consensus statements [15]. This is consistent with current criteria for postural orthostatic tachycardia syndrome (POTS), that defines orthostatic tachycardia as an increase in heart rate of 40 beats per minute within 10 min of standing for patients aged 12–18 [62]. These newer criteria provide more latitude in following children and young adolescents without other acute medical concerns in an outpatient setting. However, once children and adolescents meet medical criteria for acute stabilization, providers should act swiftly due to provide acute medical support.

Current practices for the detection and prevention of the medical complications of refeeding

Detecting and mitigating risk of medical complications during initial nutritional rehabilitation is a critical component of medical stabilization in children and young

LAB ACCESSION # JQ759427		LAB ORDER NUMBER SW0633510013145		REPORT STATUS Partially Completed	
PATIENT NAME COPELAND, C			PATIENT ID. 633511004204		
PATIENT PHONE (817) 789-9782		SEX Female	D.O.B. [REDACTED] 4/2012	AGE 13 Years	
				IS FASTING? Unknown	
COLLECTED 01/05/2026 11:14		ORDER RECEIVED BY LAB 01/06/2026 00:23			
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PHYSICIAN NAME SCHLICHTEMIER, TAMMI					
ACCOUNT: COPPELL PEDIATRICS ASSOCIATES 1705 E BELT LINE RD COPPELL, TX 75019				ACCOUNT # 63351	



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Test	Within Range	Outside Range	Units	Reference Range	Lab
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CALC BUN/CREAT	22		RATIO	6-32	MAIN
SODIUM	143		MEQ/L	133-146	MAIN
POTASSIUM	4.5		MEQ/L	3.5-5.4	MAIN
CHLORIDE	101		MEQ/L	95-107	MAIN
CARBON DIOXIDE	24		MEQ/L	19-31	MAIN
CALCIUM	10.1		MG/DL	8.4-10.2	MAIN
PROTEIN, TOTAL	7.1		G/DL	6.0-8.0	MAIN
ALBUMIN	4.8		G/DL	3.6-5.2	MAIN
CALC GLOBULIN	2.3		G/DL	2.0-3.7	MAIN
CALC A/G RATIO	2.1		RATIO	1.0-2.6	MAIN
BILIRUBIN, TOTAL		1.6 H	MG/DL	<=1.2	MAIN
ALKALINE PHOSPHATASE	196		U/L	107-384	MAIN
AST	27		U/L	9-48	MAIN
ALT	16		U/L	5-45	MAIN

LIPID PANEL

CHOLESTEROL		234 H	MG/DL	<170	MAIN
TRIGLYCERIDES		123 H	MG/DL	<90	MAIN
HDL CHOLESTEROL	62		MG/DL	>45	MAIN
CALC LDL CHOL		147 H	MG/DL	<110	MAIN

NOTE: CALCULATED LDL IS BASED ON MARTIN-HOPKINS METHOD WHICH INCLUDES ADJUSTABLE TRIGLYCERIDE:VLDL CHOLESTEROL RATIO. THIS FACTOR VARIES BY MEASURED TRIGLYCERIDE AND NON-HDL CHOLESTEROL CONCENTRATIONS WITH INCREASED CALCULATED LDL SEEN IN HIGHER TRIGLYCERIDE OR LOWER NON-HDL SPECIMENS. FOR MORE INFORMATION, SEE CLIENT ANNOUNCEMENT AT <http://www.cp1labs.com/CalcLDL-C>

RISK RATIO LDL/HDL	2.37		RATIO	<3.22	MAIN
HOMOCYSTEINE	5		UMOL/L	<8	MAIN
VITAMIN B 12 AND FOLIC ACID					
VITAMIN B-12	949		PG/ML	200-950	MAIN
FOLIC ACID	>20.0		UG/L	SEE BELOW	MAIN

INTERPRETIVE RANGES

DEFICIENCY	UG/L	<4.0
POSSIBLE DEFICIENCY.	UG/L	4.0-5.9
SUFFICIENT	UG/L	>=6.0

Cook Children's Pediatrics - Clearfork
5708 EDWARDS RANCH RD
FORT WORTH TX 76109-4115
Phone: 817-336-4040
Fax: 817-336-6780

Date: Nov 9, 2023

Physical Medicine Rehab referral

Patient: [REDACTED] Copeland
801 N. Hampton Street #1101
Fort Worth TX 76102

MRN: M002784371
DOB: [REDACTED] 012
SSN:
Sex: F

Home Phone 817-789-8498
Mobile 817-789-9782

kcopelandlaw@gmail.com

INSURANCE	PAYOR	PLAN	GROUP #	SUBSCRIBER ID	SUBSCRIBER NAME
Primary:	AETNA	AETNA POS/POS II	013373001000726	W261231917	
	AMERIGROUP STAR MCD STAR	AMERIGROUP	TXMCD000	734767215	
Secondary:					
Referring Provider Information:					
CRAWFORD, JULIE HEFLIN		Phone: 817-336-4040	Fax: 817-336-6780		

Referral Information:

Visits: 1

Urgency: Routine

Start Date: Nov 9, 2023

Referral Type: Consultation

Referral Reason: Consult and Treat

End Date: To be determined by Insurer

Diagnosis: Hypermobility Joints (M24.9)

Referred to Dept/Location: CHILDREN'S HEALTH SPECIALTY CENTER REHABILITATION

Referred to Dept/Location Address: 2222 MEDICAL DISTRICT DRIVE DALLAS, TX 75235

Referred to Dept/Location Phone Number:

Referred to Dept/Location Fax Number:

Referred to Provider:

Referred to Provider Address:

Referred to Provider Phone Number:

Referred to Provider Fax Number: 817-867-5636

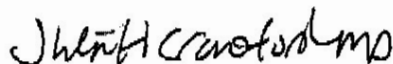
Is this referral specifically for equipment assessment? No

11 y whose mother has hypermobility Eler's Danlos Syndrome. Concern for [REDACTED] having hypermobile joints.

Signature and attestation of prescribing physician

Electronically signed by:

Date/Time: 11/9/2023 8:33 AM



Crawford, Julie Heflin, MD

NPI: 1093773053

Date/Time: 11/9/2023 8:33 AM

Attending Provider: No att. providers found

This document serves as a request of services and does not constitute insurance authorization or approval of services. To determine eligibility, please contact the members insurance carrier to verify and review coverage. If you have medical questions regarding this request for services, please contact Cook Children's Pediatrics - Clearfork at 817-336-4040 between the hours of 8:00am - 5:00pm (Mon-Fri).

Message Report



Generated: 05/04/2026 2:58 PM by Katie Copeland

Timezone: America/Chicago

Format: mm/dd/yyyy

Parents: Katie Copeland, Chris Copeland

Child(ren): [REDACTED] Copeland, [REDACTED] Copeland

Date Range: 04/30/2026 — 04/30/2026

Contains: 1 selected messages

Order: Chronological (oldest first, most recent last)

OurFamilyWizard, LLC.
ourfamilywizard.com
info@ourfamilywizard.com
+1 (866) 755-9991

Message 1 of 1

Sent: 04/30/2026 1:24 PM
From: Chris Copeland
To: Katie Copeland (*First Viewed: 05/03/2026 1:46 PM*)
Subject: Re: Sunday Visit

She doesn't have ARFID.

I will continue to follow the court orders regarding my duty to notify you about medical visits and ensure you have access to records on the portal.

Sent: 04/30/2026 1:11 PM
From: Katie Copeland
To: Chris Copeland (*First Viewed: 04/30/2026 1:15 PM*)
Subject: Re: Sunday Visit

This discussion is not optional. This is my attempt to be collaborative because the alternative if very stressful for everyone, especially the kids. However, this is too important to ignore any longer and she is running out of time.

Regarding [REDACTED] cholesterol, it's important to understand that in children with her growth profile, high cholesterol is often a clinical sign of metabolic compensation for malnutrition. It indicates that her endocrine system is under stress. Treating the cholesterol without addressing the underlying nutritional deficit and ARFID is treating a symptom while the root cause—which is causing her growth stunting—remains unaddressed.

Here is a quick explanation:

App. 23

When a child presents with high cholesterol (hyperlipidemia) alongside significant growth stunting and low weight (1st-3rd percentiles), it often signals that the body's endocrine system is compensating for a lack of nutritional intake.

In cases of chronic malnutrition or ARFID, the body enters a "starvation state" to preserve energy. This metabolic shift can trigger several endocrine-related issues:

1. Metabolic Adaptation

When the body doesn't receive enough fuel, the liver's processing of lipids can change. This can cause "paradoxical hypercholesterolemia," where cholesterol levels rise because the body is breaking down its own fat stores for energy or because the thyroid is slowing down to conserve calories.

2. Thyroid Suppression (Sick Euthyroid Syndrome)

Malnutrition often leads to a decrease in thyroid hormone production (T3 and T4). Since thyroid hormones are responsible for clearing LDL (bad cholesterol) from the blood, a slowed metabolism directly leads to higher cholesterol readings. This isn't a "cholesterol problem"—it's a "starvation response."

3. Growth Hormone Resistance

The stunting we are seeing suggests that the IGF-1 pathway is being affected. Even if her body is producing growth hormone, malnutrition makes the body "resistant" to it, prioritizing immediate survival over physical growth.

Sent: 04/30/2026 10:47 AM
From: Chris Copeland
To: Katie Copeland (First Viewed: 04/30/2026 12:58 PM)
Subject: Re: Sunday Visit

I don't recall you attempting to make any agreement about the visit this weekend. This is the first message about it you've sent me.

However, pending confirmation from Cindy, then that timing will be agreeable.

 has been thriving. I have been following the recommendations by the doctors regarding her cholesterol. There is nothing to discuss.

Sent: 04/30/2026 10:35 AM
From: Katie Copeland
To: Chris Copeland (First Viewed: 04/30/2026 10:36 AM)
Subject: Sunday Visit

Hey — just confirming that Sunday at my place from 10:00 AM–1:00 PM is what we agreed to. Let me know if anything needs to change.

Also, can we set up a time for a call this week? I'm fine with Susan being included if that's

App. 24

easier.

I have serious concerns about [REDACTED] health, specifically her food-related issues, and I think we need to act quickly to get her the right support while there's still time for interventions to make a real difference. I'd like us to align on:

- what symptoms/concerns we're each seeing,
- what evaluation/treatment options we're pursuing,
- and what the next concrete steps are.

Please send a few windows when you can talk (or I can propose times).

Katie

Automated Certificate of eService

This automated certificate of service was created by the eFiling system. The filer served this document via email generated by the eFiling system on the date and to the persons listed below. The rules governing certificates of service have not changed. Filers must still provide a certificate of service that complies with all applicable rules.

Envelope ID: 114462593

Filing Code Description: Notice Of Filing

Filing Description: PETITIONER'S REQUEST FOR PERMISSION TO FILE AN EMERGENCY MOTION FOR TEMPORARY ORDERS

Status as of 5/6/2026 4:17 PM CST

Case Contacts

Name	BarNumber	Email	TimestampSubmitted	Status
Susan Smith		SUSAN@SUSANSMITHFAMILYLAW.COM	5/4/2026 4:56:52 PM	SENT
Kathryn Copeland		[REDACTED]@gmail.com	5/4/2026 4:56:52 PM	SENT
André Smith		paralegal@susansmithfamilylaw.com	5/4/2026 4:56:52 PM	SENT
Christopher Copeland		cjcopeland@gmail.com	5/4/2026 4:56:52 PM	SENT
Kathryn Copeland	24086056	Katie@CopelandLawTexas.com	5/4/2026 4:56:52 PM	SENT

IN THE SUPREME COURT OF TEXAS

No. _____

IN RE KATHRYN COPELAND, RELATOR

On Original Proceeding from the Second Court of Appeals,
Fort Worth, Texas — No. 02-24-00278-CV
Arising from No. 231-686108-20, 231st Judicial District Court,
Tarrant County, Texas

APPENDIX

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***Copeland v. Tarrant County* — ADA Title II Case Summary and
Tarrant County Written Admissions (April–May 2026).**

County states court-user accommodations are within judges' sole discretion;
no Title II compliance records located in response to open-records request.

Federal Fifth Circuit Court of Appeals, New Orleans — No. 26-10389

COPELAND v. TARRANT COUNTY

ADA Title II / § 504 Court-Access Infrastructure Case

Fifth Circuit emergency relief denied by divided panel; Judge Haynes would have granted temporary relief

EMERGENCY SCOTUS APPLICATION IN PREPARATION

THE QUESTION

Whether federal courts may decline to enforce ADA Title II / § 504 court-access infrastructure requirements when the public entity cannot identify the required accommodation pathway and the plaintiff seeks process-based relief rather than a ruling on state-court merits.

Related issue: what standard applies to ADA interference claims under 42 U.S.C. § 12203(b) in the Fifth Circuit.

§ 504 / FEDERAL FUNDING HOOK

Title II applies because courts and counties are public entities. § 504 adds a federal-funding hook where programs receive federal financial assistance.

The OAG Child Support Division operates through federally funded Title IV-D enforcement machinery, raising an enforcement-integrity question: whether federally funded enforcement may proceed on records generated through proceedings lacking a functioning disability-access pathway.

WHY IT MATTERS

- Tarrant County is Texas's third-largest county, with more than 2 million residents.
- Federal law has required public entities to maintain ADA Title II compliance infrastructure for decades, including self-evaluation, public notice, ADA coordination, and grievance procedures.
- Tarrant County stated it was unable to locate responsive records identifying a Title II coordinator, grievance procedure, self-evaluation/compliance audit, or court-user accommodation process for judicial proceedings.
- County officials have also stated that court-user accommodation issues are left to individual judges and are not the County's ADA responsibility.
- Fifth Circuit emergency relief was denied by a divided panel; Judge Haynes would have granted temporary relief.
- The requested relief is procedural: a functioning access pathway and written accommodation determinations before further inaccessible proceedings or enforcement actions continue.

TARRANT COUNTY DOCUMENTED WRITTEN RECORD

- **Apr. 1, 2026:** Assistant County Administrator stated court-proceeding accommodation requests are not within County ADA compliance, are left to judges, and the matter is considered closed.
- **Apr. 21, 2026:** Open Records Response CDA-2026-897 stated the County was unable to locate responsive Title II ADA records after contacting departments and individuals.
- **Apr. 30, 2026:** Tarrant County Criminal District Attorney's Office stated the County has no control over judicial accommodation decisions and did not view corrective action as required.
- **Post-notice issue:** after being placed on notice, Tarrant County continued to disclaim responsibility rather than identify or establish a court-user ADA pathway.

APPLICANT

- Kathryn "Katie" Copeland is a Texas-licensed attorney, pro se litigant, and author of *Access Is Not Advantage: Structural Integrity and Disability Accommodation in State Courts*, published by the ABA Commission on Disability Rights in 2026.
- SSA issued a fully favorable disability decision on Apr. 10, 2026, with established onset June 8, 2023, predating the relevant state-court proceedings. Applicant has documented neurological/cognitive disability affecting meaningful participation without accommodations.

CHILDREN'S INDEPENDENT HARM

The access failure does not affect only the disabled parent. Orders and enforcement actions generated without meaningful participation can impair children's relationship, stability, and access to their parent. This supports irreparable-harm and associational-discrimination analysis under Title II / § 504 without asking any federal court to decide custody merits.

RELIEF SOUGHT: PROCESS, NOT MERITS

Pause or condition further Tarrant County proceedings and enforcement actions requiring Applicant's participation until a functioning ADA Title II / § 504 pathway exists, including public notice, a designated coordinator, a grievance procedure, and written determinations on accommodation requests.

The requested relief is procedural, not substantive. It does not ask any court to decide custody, support, housing, or other state-law merits.

WHAT IS NEEDED

- Emergency appellate / SCOTUS triage
- Co-counsel, amicus, or referral support
- Disability-rights organizational review
- Research support on *Sprint*, *Younger*, Title II, § 504, and § 12203(b)
- Investigative review of Tarrant County's documented court-access gap

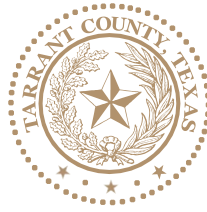
CONTACT

Kathryn "Katie" Copeland, J.D. Texas Bar No. 24086056
(817) 789-8498 | Katie@KatieCopeland.com

KatieCopeland.com | American Bar Association publication:

[*Access Is Not Advantage: Structural Integrity and Disability Accommodation in State Courts*](#), April 2, 2026.

Prepared as an outreach fact sheet. Source documents and court orders available on request.



PHIL SORRELLS
Criminal District Attorney
Tarrant County

April 30, 2026

Ms. Kathryn Copeland
Copeland Law
1137 S. Pine Street
Grapevine, Texas 76051

Via E-Mail

Re: Letter dated April 20, 2026 to the County Administrator

Ms. Copeland,

I am in receipt of your correspondence dated April 20, 2026, which was addressed to County Administrator Chandler Merritt. After reviewing this letter and the attached exhibits, I understand you have concerns regarding the provision of ADA accommodations during the course of your family court judicial proceedings.

First, I understand that you requested ADA accommodations from Tarrant County District Court Judges and were dissatisfied with their decisions. As the County communicated to you upon your initial request to the County for judicial ADA accommodations, the County does not have control over a particular state District Court Judge's decision when a litigant seeks ADA accommodations that would modify court proceedings. Such a decision relates to a judge's adjudicative responsibilities and is within a judge's sole discretion. Tarrant County, on the other hand, provides assistance for those with disabilities that prevent them from accessing and navigating the County's physical facilities, programs, services, and activities, whether in person or virtually. While I understand you are frustrated and feel the process is fragmented, your case took place in a state District Court that is located within, but not controlled by, Tarrant County. Thus, concerns with the state court ADA accommodation process are not properly presented to Tarrant County because the County cannot control or change the outcome of your legal proceedings.

Your letter also requests certain documents. It is my understanding that you submitted an open records request that mirrors your request for records in your April 20 correspondence, and

that the open records request was closed on or about April 21, 2026. Thus, you have received the records in the County's possession that are responsive to your request.

Finally, you expressed concerns with the County's Title II compliance. Tarrant County takes its obligations under Title II seriously and remains committed to ensuring all individuals, including those with disabilities, have access to the County. As your frustrations relate to judicial decisions and do not outline any issues accessing County facilities or programs, it does not appear that the County is required to take corrective action based on the allegations you presented.

Thank you for giving us an opportunity to review and address your concerns. If you have any additional questions regarding the County's ADA policies, please contact me directly.

Sincerely,

Phil Sorrells
Criminal District Attorney
Tarrant County, Texas

Katherine E. Owens

KATHERINE E. OWENS
Assistant Criminal District Attorney

KEO/psm

Re: Re: Tarrant County District Attorney's Office - Records Request CDA-2026-897 Completed

From Paul Nguyen (Tarrant County, TX) <TarrantCountyTX@request.justfoia.com>

To katie@copelandlawtexas.com

CC pkguyen@tarrantcountytexas.gov

Date Tuesday, April 21st, 2026 at 4:28 PM

Ms. Copeland,

Thank you for following up. We have reached out to various departments and individuals regarding the Title II ADA aspects of your request and unfortunately, we were still unable to locate any responsive information that you are seeking.

Tarrant County District Attorney's Office
401 W. Belknap
Fort Worth, Texas 76196
(817) 884-1233
openrecords@tarrantcountytexas.gov

On Mon, Apr 20, 2026 at 4:25 pm, Katie Copeland wrote:

Thank you. However, I was not referring to the employment aspect of ADA, but rather the Title II aspect of ADA, which applies to counties and courthouses (under the US Supreme Court case of Tennessee v Lane and the 5th Circuit opinion from Luke v Texas). For example, I have attached the City of Fort Worth's compliance documents.

For convenience, I have pasted my original request again below:

Request #CDA-2026-897

I request access to and copies of the following records:

1. ADA Title II Self- Evaluation (28 C.F.R. § 35.105) All records reflecting any self-evaluation conducted by Tarrant County and/or its courts in compliance with 28 C.F.R. § 35.105, including but not limited to: • The original self-evaluation required to be completed by January 26, 1993 • Any subsequent updates, revisions, or re-evaluations • All supporting materials associated with such evaluations
2. Required Components of the Self- Evaluation To the extent maintained, please include records showing: • Lists of individuals and organizations consulted (including disability advocacy groups) • Descriptions of areas examined and barriers identified • Descriptions of modifications made as a result of the evaluation
3. ADA Compliance Infrastructure Records sufficient to show: • The designation of any ADA Coordinator pursuant to 28 C.F.R. § 35.107 • Any ADA grievance procedures maintained by the County or its courts • Policies or procedures related to ADA Title II compliance in court operations
4. Record Retention and Availability • Any record retention policies applicable to ADA compliance documents, including self-evaluations • Any documentation explaining the absence, destruction, or unavailability of responsive records, if applicable _____

If there are no documents or individuals related to these request, please confirm.

Sincerely,
Kathryn "Katie" Copeland

Re: ADA Justice Issue

From Travis H. Yarbrough <thyarbrough@tarrantcountytx.gov>

To katie@copelandlawtexas.com

Date Wednesday, April 1st, 2026 at 3:03 PM

The presiding Judge over each court is their own decision maker with regards to court proceedings. I am not aware of how each court handles their request once they get notified.

Since it is a court proceeding request and not falling under the County's ADA compliance the response and directing them to the court then considers the matter closed.



Travis Yarbrough

Assistant County Administrator

100 E Weatherford St. Suite 404

Fort Worth, TX 76196

Phone: 817-884-2643

Email: thyarbrough@tarrantcountytx.gov

From: Katie Copeland <katie@copelandlawtexas.com>

Sent: Monday, March 30, 2026 4:13 PM

To: Travis H. Yarbrough <thyarbrough@tarrantcountytx.gov>

Subject: Re: ADA Justice Issue

EXTERNAL EMAIL ALERT! Think Before You Click!

Hi Mr. Yarbrough,

Thank you for your response.

This is related in part to my earlier request, but it also concerns an ongoing and active access issue in a current civil matter in the 141st District Court (Cause No. 141-370402-25).

The issue I am encountering is not limited to a single court, but appears to involve a broader gap in how ADA accommodation requests are received, routed, and resolved for court users at the county level.

In my current case, I have made repeated requests for accommodations necessary for meaningful participation due to documented cognitive limitations. However, there has been no clear ADA coordination pathway, no identified decision-maker, and no interactive process through which those requests are evaluated.

As a result, proceedings have continued while access issues remain unresolved.

At this point, I am trying to identify:

- who serves as the appropriate ADA decision-maker for court-user accommodations at the county level;

- how requests are supposed to be processed across courts; and
- how an interactive process is initiated and documented.

If this falls within your scope, I would appreciate your guidance. If not, I would be grateful if you could direct me to the appropriate individual responsible for ADA coordination in this context.

I appreciate your time and your help in clarifying this.

Best regards,

Kathryn "Katie" Copeland
(817) 789-8498

On Friday, March 27th, 2026 at 2:17 PM, Travis H. Yarbrough <thyarbrough@tarrantcountytx.gov> wrote:

Good afternoon Ms. Copeland,

I was given a message that you reached out to our office yesterday concerning an ADA question. I did find a request from you that went to Justice of the Peace court No. 3 earlier this year. Does this happen to be in relation to that request or something entirely different?



Travis Yarbrough

Assistant County Administrator

100 E Weatherford St. Suite 404

Fort Worth, TX 76196

Phone: 817-884-2643

Email: thyarbrough@tarrantcountytx.gov

38.07 KB 2 embedded images

Outlook-eb1xy3s4.png 19.04 KB

Outlook-flwgoutp.png 19.04 KB